

The Complex SRHR Triad

A report on the Intersecting Burdens of Gender-Based Violence, Unintended Pregnancy, and
Post-Abortion Care in Eswatini



Journey Of Hope
For Girls and Women Eswatini

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Violence, Unintended Pregnancy, and
Post-Abortion Care in Eswatini

**1 BILLION
RISING
Eswatini**

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Director's Note



As the Director of Journey of Hope for Girls and Women in Eswatini, it is both a professional duty and a personal burden to present this research report. This report is not merely a collection of data points; it is a mirror reflecting the lived realities of thousands of young women and girls across our Kingdom who navigate a landscape of intersecting vulnerabilities every day.

Our study was born from a simple but urgent question: Why, despite our national efforts, do so many young lives remain trapped in a cycle of violence and reproductive crisis? The findings provide an outcome which uncovers what we term the Complex SRHR Triad; a concerning intersection where Gender-Based Violence (GBV), Early Unintended Pregnancy (EUP), and the systemic barriers to Post-Abortion Care (PAC) collide to present an SRHR burden that disempowers adolescent girls and young women.

The weight of this study is found in the numbers that can no longer be ignored:

- The Geographic Divide - In our rural communities, a staggering 82.8% of pregnancies are unintended, compared to 69.6% in urban areas, highlighting a deep-seated crisis of access and autonomy in our most vulnerable regions.
- The Violence Link - We found that 72.5% of unplanned pregnancies in urban settings occurred within relationships characterized by Intimate Partner Violence (IPV). In rural areas, 20.7% of unintended pregnancies were the direct result of rape or sexual violence.
- The Crisis of the Minor - For girls under the age of 18, the situation is a national emergency 93.3% of their pregnancies are unplanned, and 21% of all pregnancies studied involve minors, many of whom are survivors of statutory rape.
- The Confidentiality Trap - Perhaps most concerning is that 86.2% of urban respondents expressed a profound fear of legal repercussions, while over 73% of adolescents perceive healthcare providers as hostile or judgmental.

To this end, this report is a call to all stakeholders from the Ministry of Health, Ministry of Education, Department of Social Welfare, our parliament and Ministry of Justice to our Traditional Leaders and Civil Society partners to address challenges highlighted above. We cannot continue to treat these issues in silos therefore we must unite to dismantle the legal and social gatekeeping that disadvantages our adolescent girls and young women. We must bridge the gap between our constitutional promises and the clinical reality on the ground. The future of Eswatini's demographic dividend depends on our courage to reform. I invite you to engage with this evidence not with judgment, but with a shared commitment to ensuring that every girl in Eswatini can exercise autonomy over her body and her future.

Director, Journey of Hope for Girls and Women in Eswatini

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We are grateful for the cooperation and guidance provided by the Government of Eswatini. Specifically, we thank the Ministry of Health, the Ministry of Education, and the Department of Social Welfare for their roles in situating these findings within the national development framework. We also acknowledge the Police of the Royal Kingdom of Eswatini for their perspectives on the legal complexities surrounding GBV and the protection of minors. Special recognition is due to the SADC Parliamentary Forum (SADC PF) Representative for providing the regional context and emphasizing the necessity of domesticating model laws to protect the rights of women and girls across Southern Africa.

This research would not exist without the people of Eswatini. We thank the Traditional and Community Leaders for opening the doors to their jurisdictions, and the members of the Bhunya, Ngonini, and Matsapha communities for welcoming our team and sharing their lived realities. Most importantly, we thank the parents and guardians who entrusted us with their consent, and the young women and girls who spoke their truths. We also honor the dedicated

health service providers who shared their professional challenges in a complex legal landscape.

Heartfelt appreciation goes to the co-investigators of this study, Nyasha Sengayi (OBR Zimbabwe) and Colani Hlatjwako (Journey of Hope/OBR Eswatini). We specifically thank Nyasha Sengayi for her work in drafting this report and synthesizing complex data into a powerful narrative for reform.

Finally, we acknowledge the staff of Journey of Hope for Girls and Women for their exceptional logistical support and unwavering dedication to the safety and success of this field research.



ACRONYMS

WAGYW – Adolescent Girls and Young Women
ART – Antiretroviral Therapy
CEDAW – Convention on the Elimination of All Forms of Discrimination Against Women
CRC – Convention on the Rights of the Child
CSO – Civil Society Organization
D&C – Dilation and Curettage (a surgical procedure used for clearing the uterine lining)
DPMO – Deputy Prime Minister's Office
EHRRB – Eswatini Health and Human Research Review Board
EUP – Early Unintended Pregnancy
FP – Family Planning
GBV – Gender-Based Violence
IPV – Intimate Partner Violence
KAP – Knowledge, Attitude, and Practice
LGBTQI+ – Lesbian, Gay, Bisexual, Transgender, Queer, Intersex, and others
MMR – Maternal Mortality Ratio
MoE – Ministry of Education
MoH – Ministry of Health
MoJ – Ministry of Justice
MVA – Manual Vacuum Aspiration (a WHO-recommended method for managing abortion complications)
OBR – One Billion Rising
PAC – Post-Abortion Care
PI / Co-PI – Principal Investigator / Co-Principal Investigator
PWD – Persons with Disabilities
SADC – Southern African Development Community
SADC PF – Southern African Development Community Parliamentary Forum
SDG – Sustainable Development Goal
SOP – Standard Operating Procedure
SRHR – Sexual and Reproductive Health and Rights
WHO – World Health Organization, and the young women and girls who spoke their truths. We also honor the dedicated

EXECUTIVE SUMMARY

This report, commissioned by Journey of Hope for Girls and Women in Eswatini under the One Billion Rising Campaign, presents the findings of a baseline assessment investigating the linked burdens of Gender-Based Violence (GBV), Early Unintended Pregnancy (EUP), and the systemic barriers to integrated Post-Abortion Care (PAC). In a national context where maternal mortality remains high and approximately one in three women experience sexual abuse before the age of 18, this study serves as a critical evidentiary baseline to inform policy reform, health system strengthening, and community-led advocacy.

The study utilized a Comparative Knowledge, Attitude, and Practice (KAP) design, surveying 260 participants across three primary sites: Bhunya, Ngonini, and Matsapha. The sample was predominantly young, with the majority falling within the 18–26 age cohort, representing both urban (75.4%) and rural (24.6%) populations. This quantitative data was strengthened by community focus group discussions, qualitative key informant interviews with policy makers, SADC Parliamentary Forum researcher, and Civil Society Organizations (CSOs) to provide a holistic view of the structural and socio-cultural barriers to Sexual and Reproductive Health and Rights (SRHR).

The research uncovered a correlation between violence and reproductive outcomes. A staggering 72.5% of unplanned pregnancies in urban settings occurred within relationships characterized by Intimate Partner Violence (IPV), while in rural areas, 20.7% of unintended pregnancies were the direct result of rape or sexual violence. The vulnerability of minors is particularly acute, with 93.3% of pregnancies among girls under 18 being unplanned, and 21% of the total pregnancy prevalence in the study involving minors a statistic that highlights the pervasive issue of statutory rape.

Furthermore, the study established a proxy incidence for unsafe abortion. Despite the legal restrictions, the combination of high EUP rates, significant fear of legal repercussions (86.2%), and widespread perceptions of healthcare provider hostility (73%) suggests that many young women resort to clandestine, unsafe procedures. This creates what the report terms the Loophole of Unintentional Aid, where the state, by failing to provide safe and legal reproductive services, ends up paying a much higher fiscal and social price to treat the medical complications arising from the backyard abortion industry.

Two primary barriers emerged as significant obstacles to care:

- The Confidentiality Trap – A conflict between the clinical duty of care and the legal mandate to report minors and cases of abortion. Current practices prioritize reporting over patient privacy, effectively deterring adolescents from seeking life-saving PAC and SRHR Services for fear of criminal involvement or social shaming.
- Socio-Cultural Gatekeeping – In rural areas, the influence of traditional and male leadership is absolute. The study found that 100% of rural respondents acknowledge the power of traditional structures over their reproductive decisions, often leading to intersectional invisibility for marginalized groups, including the LGBTQI+ community and persons with disabilities.

The report concludes with a draft comprehensive Action Plan designed to move Eswatini from a punitive reproductive health framework to one rooted in human rights and public health efficiency.

- Immediate Clinical Integration – The Ministry of Health must operationalize a one-stop-shop model where family planning and PAC are integrated into existing HIV/ART services.
- Legislative Reform – Immediate action is required to operationalize Section 15(5) of the Constitution through clear clinical guidelines. Furthermore, the Children's Protection and

Welfare Act must be amended to provide medical officers with immunity from mandatory reporting in cases of abortion complications, prioritizing health over criminalization.

- Fiscal Accountability – The government must establish a dedicated SRHR Budget Allocation to ensure the sustainable supply of commodities and reduce reliance on unpredictable donor funding.
- Intersectional Inclusion– Specific protocols must be developed to address the triple Stigma faced by LGBTQI+ individuals and persons with disabilities, ensuring that their specific vulnerabilities are addressed without judgment or coercion.

The findings of this baseline assessment underscore that the current SRHR landscape in Eswatini is a public health emergency. The convergence of restrictive laws, pervasive sexual violence, and a health environment disproportionately harms the nation's youth. To achieve the 2030 Sustainable Development Goals and the African Union's Agenda 2063, Eswatini must courageously dismantle these barriers, foster community dialogue with traditional leaders, and recognize reproductive autonomy as a fundamental pillar of national stability and human dignity.



1. SECTION A: Introduction

The Kingdom of Eswatini is currently navigating a complex epidemiological and demographic transition characterized by a youthful population structure and persistent challenges in the realization of sexual and reproductive health rights (SRHR). With nearly half of the population (46.9%) under the age of 25, the state's capacity to provide integrated, rights-based reproductive health services is a critical determinant of its long-term socio-economic stability and its ability to achieve the goals set forth in the African Union's Agenda 2063. The intersection of a highly restrictive legal framework, pervasive gender-based violence (GBV), and systemic failures in the public health sector has created an environment where adolescent girls and young women (AGYW) face extreme risks related to early unintended pregnancy (EUP) and the complications of unsafe abortion.

1.1 The Public Health Triad: GBV, EUP, and PAC

The realization of reproductive rights in Eswatini is hindered by the intersection of gender-based violence, early unintended pregnancy, and barriers to integrated post-abortion care. GBV is not merely a social issue but a primary upstream determinant of poor health outcomes. National statistics reveal that 25.5% of females aged 13–24 have experienced violence, and approximately 1 in 3 women experience sexual abuse before age 18. This violence severely restricts reproductive autonomy, as survivors are often unable to negotiate contraceptive use or decide on childbearing. Intimate partner violence (IPV) is also particularly prevalent, with half of all women reporting abuse by a current or former partner. The statistical association between this violence and unintended pregnancy is strong; studies in the region show that sexual violence significantly increases the odds of mistimed or unwanted conceptions. This high prevalence of violence is directly connected to reproductive health outcomes. Scientific literature, including studies conducted in Southern Africa, establishes a statistical association between experiences of sexual or intimate partner violence and increased likelihood of unintended pregnancy. When unintended pregnancies occur, the lack of safe termination options leads to the second pillar of the triad: unsafe

Prevalence of risky sexual behaviour and its association with alcohol use among unmarried adolescents and young adults in Eswatini: Evidence from a population-based survey | medRxiv, accessed on February 6, 2026, <https://www.medrxiv.org/content/10.1101/2025.11.24.25340940v1.full-text>

Key Interview with CSO organisation

eswatini – COUNTRY PROFILE, accessed on February 6, 2026,

https://www.yplusglobal.org/docs/resources/HER_Voice_Fund_Country_Profiles_2024/HER_Voice_Fund_Country_Profile_Eswatini_2024.pdf

Ending gender-based violence and all harmful practices – UNFPA Eswatini, accessed on February 6, 2026,

<https://eswatini.unfpa.org/en/topics/ending-gender-based-violence-and-all-harmful-practices>

The Relationship Between Intimate Partner Violence and Unintended Pregnancy: Analysis of a National Sample From Colombia | Guttmacher Institute, accessed on October 5, 2025,

<https://www.guttmacher.org/journals/ipsrh/2004/relationship-between-intimate-partner-violence-and-unintended-pregnancy>



abortion. With an estimated annual burden of 30,000 to 34,000 unintended pregnancies, the demand for reproductive options in Eswatini is immense, yet unmet need for family planning remains high at over 20%. The third pillar, post-abortion care (PAC), is intended as a life-saving strategy to manage complications from spontaneous or induced pregnancy loss. However, PAC in Eswatini is hindered by a lack of set operational standards even though if patients present with complications, they can get assistance.

1.2 Socio-Economic Context and SRHR Imperatives

The socio-economic landscape of Eswatini profoundly shapes reproductive outcomes. While the country has made significant strides in reducing AIDS-related deaths through expanded Test and Treat programs, HIV prevalence remains the highest globally, at approximately 24.8% among the adult population. This epidemic does not exist in isolation; it is deeply intertwined with reproductive health challenges. Females continue to exhibit higher HIV prevalence and incidence compared to males, particularly in younger age bands, often due to a lack of power over their own sexual health and decision-making.

Demographic data for 2025 projects a total population of approximately 1.3 million, with a total fertility rate (TFR) that has gradually declined from 4.0 in 2007 to 2.7 in 2025. Despite this overall decline, adolescent fertility remains disproportionately high, with 69 to 87 births per 1,000 girls aged 15–19. This rate is significantly elevated among adolescents from the lowest wealth quintile, where the birth rate reaches 122 per 1,000, illustrating the sharp divide between economic vulnerability and reproductive risk.

While international agencies like the World Bank and WHO have reported improvements from 580 deaths per 100,000 live births in 2000 to approximately 118 in 2023, national administrative data

Executive Board of the United Nations Development Programme, the United Nations Population Fund and the United Nations Office for Women, accessed on February 6, 2026, <https://www.unfpa.org/sites/default/files/board-documents/main-document/DP.FPA.CPD.SWZ.8%20-%20Eswatini%20CPD%20-%20Final%20-%202027Jun25.pdf>

Abortion in Eswatini – Wikipedia, accessed on February 6, 2026, https://en.wikipedia.org/wiki/Abortion_in_Eswatini

Confirmed by interviews with Health Service providers from different locations
Sexual & reproductive health – UNFPA Eswatini, accessed on February 6, 2026, <https://eswatini.unfpa.org/en/topics/sexual-reproductive-health-5>

UNFPA Kingdom of Eswatini | National context, accessed on February 6, 2026, <https://eswatini.unfpa.org/en/national-context-0>

Confirmed by Key Interview with SAFAIDS representative
Ibid,8

Eswatini – Gender Data Portal – World Bank, accessed on February 6, 2026, <https://genderdata.worldbank.org/en/economies/eswatini>

Supra note 8

eSwatini – Gender Data Portal – World Bank, accessed on February 6, 2026, <https://genderdata.worldbank.org/en/economies/eswatini> and confidential enquiries into maternal deaths suggest that the burden remains much higher

in specific underserved regions, with figures reaching up to 452 per 100,000. A critical driver of this mortality is the high incidence of unsafe abortions, which is estimated to be the third leading cause of maternal death in the country.

The magnitude of unintended pregnancy establishes a massive underlying demand for either expanded contraceptive services and accessible, safe abortion care. Estimates suggest an annual burden of unintended pregnancies ranging between 30,000 and 34,000 cases, with a significant proportion of these occurring among adolescents. The Adolescent Birth Rate (ABR) remains alarmingly high at 87 births per 1,000 girls aged 15–19, and approximately 25% of all institutional deliveries are attributed to this age group.

1.3 Family Planning Access, Unmet Need, and Implications for Early Unintended Pregnancy

Access to effective family planning services remains a central determinant of reproductive health outcomes in Eswatini. Although the country has historically recorded relatively high levels of contraceptive use compared to several countries in the Southern African region, recent evidence suggests fluctuations in prevalence, persistent reliance on short-acting methods, and widening gaps in access among certain population groups. Current estimates indicate that the modern contraceptive prevalence rate (mCPR) among all women of reproductive age is approximately 48.3% as of 2025. While this level reflects substantial coverage, important disparities are evident when contraceptive use is disaggregated by marital status. Among married women, modern contraceptive use declined from 66.1% in 2014 to approximately 58% by 2022, suggesting a reversal of earlier gains. By contrast, contraceptive use among unmarried women has increased

UNFPA Kingdom of Eswatini | National context, accessed on February 6, 2026, <https://eswatini.unfpa.org/en/national-context-0>

RESILIENCE IN ACTION: – UNFPA Eswatini, accessed on February 6, 2026, <https://eswatini.unfpa.org/sites/default/files/pub-pdf/2025-08/UNFPA%20Eswatini%20Annual%20Report%202024.pdf>

Interview with policy maker from ministry of Health

Prevalence of risky sexual behavior and its association with alcohol use among unmarried adolescents and young adults in Eswatini: Evidence from a population-based survey | medRxiv, accessed on February 6, 2026, <https://www.medrxiv.org/content/10.1101/2025.11.24.25340940v1.full-text>

Ibid, 15

FP2030/Track20, 2025. Eswatini 2025 Country Fact Sheet. Available at: <https://www.track20.org>
WHO Regional Office for Africa, 2023. Eswatini Strategic Assessment on Unintended Pregnancies, Contraception and Post-Abortion Care.



modestly from 38% in 2012 to roughly 42% by 2023, reflecting a gradual expansion of services targeting sexually active unmarried women.

The contraceptive method mix in Eswatini is heavily skewed toward short-acting methods that require consistent user adherence and regular resupply. Male condoms account for approximately 46% of contraceptive use, making them the most commonly used method, particularly among adolescents and young adults. Injectable contraceptives represent roughly 32%, while oral contraceptive pills account for approximately 12%. In contrast, uptake of long-acting reversible contraceptives (LARCs) remains comparatively low. Implants account for about 17.8% of contraceptive use, while intrauterine devices (IUDs) represent only around 2.5%, indicating limited adoption of highly effective long-term methods. This reliance on short-acting methods increases the risk of contraceptive discontinuation, incorrect use, or method failure, thereby elevating the likelihood of unintended pregnancy.

Despite the relatively high contraceptive prevalence, unmet need for family planning remains a persistent challenge. Unmet need refers to women who wish to delay or avoid pregnancy but are not using any contraceptive method. National estimates suggest that unmet need for modern contraception among all women stands at approximately 12.1% in 2025. However, survey data indicate higher levels among specific sub-populations. Among married women, unmet need increased from 15% in 2014 to approximately 20.4% in 2021–2022, suggesting that improvements in access have not been consistently sustained.

Young women remain particularly vulnerable. Among women aged 20–24 years, unmet need increased sharply from 17.4% in 2014 to 30.3% in 2022, highlighting persistent barriers to

Frontiers in Reproductive Health, 2025. Contraceptive use among adolescent girls and young women ages 15–24 in seven high HIV prevalence countries.

FP2030/Track20, 2025. Eswatini 2025 Country Fact Sheet. Available at: <https://www.track20.org>

UNFPA, 2025. Eswatini Country Programme Document (2026–2030). Executive Board of UNDP, UNFPA and UNOPS.

UNFPA Eswatini, 2025. Ending Unmet Need for Family Planning. Available at: <https://eswatini.unfpa.org>

WHO Regional Office for Africa, 2023. Eswatini Strategic Assessment on Unintended Pregnancies, Contraception and Post-Abortion Care.

contraceptive access among adolescents and young adults. These disparities are further shaped by geographic and socio-economic inequalities. Women in rural areas experience higher unmet need (23%) compared to 14.3% among urban women, while women in the lowest wealth quintiles face an unmet need rate of approximately 24.3%, reflecting the influence of poverty and service accessibility on reproductive health outcomes.

These gaps in contraceptive access have direct implications for the prevalence of early unintended pregnancies (EUPs) in Eswatini. When women particularly adolescents and young women are unable to access reliable contraception or rely on short-acting methods prone to discontinuation or incorrect use, the likelihood of unintended pregnancy increases significantly. Evidence from regional studies demonstrates that unmet need for contraception and reliance on less effective methods are among the strongest predictors of unintended pregnancy among young women in high HIV prevalence settings. In the context of Eswatini, where adolescents and young women already face heightened vulnerability due to gender inequality, limited reproductive autonomy, and stigma surrounding sexual health services, gaps in contraceptive access contribute directly to higher rates of unintended pregnancy.

In settings where access to safe and legal abortion services is restricted or poorly understood, unintended pregnancies often lead women to seek clandestine or unsafe methods to terminate pregnancies. Consequently, unmet need for family planning functions as an upstream driver of unsafe abortion, particularly among young women who may face social stigma, financial barriers, or limited awareness of available reproductive health services. Studies across sub-Saharan Africa consistently show that where contraceptive access is constrained, unintended pregnancy and unsafe abortion rates increase concurrently. Within the broader framework of this report, these dynamics reinforce the interconnected nature of the SRHR triad (gender-based violence, early unintended pregnancy, and unsafe abortion) where structural barriers to reproductive health services exacerbate vulnerability across multiple dimensions of women's health and rights.

1.4 Study Rationale

Frontiers in Reproductive Health, 2025. Contraceptive use among adolescent girls and young women ages 15–24 in seven high HIV prevalence countries.

UNFPA, 2025. Eswatini Country Programme Document (2026–2030). Executive Board of UNDP, UNFPA and UNOPS.

Frontiers in Reproductive Health, 2025. Contraceptive use among adolescent girls and young women ages 15–24 in seven high HIV prevalence countries.

WHO Regional Office for Africa, 2023. Eswatini Strategic Assessment on Unintended Pregnancies, Contraception and Post-Abortion Care.



The study rationale for this report is rooted in the urgent need to address the intersecting crises of gender-based violence (GBV), early unintended pregnancy (EUP), and the systemic barriers to life-saving post-abortion care (PAC) in Eswatini. As the nation navigates a demographic transition where nearly half the population is under the age of 25, the gap in providing, rights-based reproductive health services pose a direct threat to socio-economic stability and the health of adolescent girls and young women. Despite significant progress in combating the HIV epidemic, maternal mortality remains a critical concern, with unsafe abortions identified as the third leading cause of maternal death.

The current legal and social environment creates a complex situation where reproductive autonomy is severely restricted by a restrictive legal framework and prevalent patriarchal influence. National statistics underscore the gravity of the situation, showing that approximately one in three women experience sexual abuse before age 18, which directly fuels the high rates of unintended pregnancy. Furthermore, a confidentiality trap exists within the health system, where legal requirements for medical staff to report minors override patient privacy, effectively deterring the most vulnerable from seeking professional care.

This study aims to quantify these linkages and evaluate the health system's readiness to provide integrated services. The research provides the necessary evidence to advocate for policy reforms that align with international standards. Ultimately, the rationale is to bridge the gap between high levels of health literacy and low levels of health-seeking behavior caused by fear of legal sanction and social stigma.

1.5 Study Objectives

1. Objective 1 (GBV-EUP Linkage) – To quantify the prevalence of Gender-Based Violence (GBV), particularly sexual and intimate partner violence, and assess its association with the prevalence of Early Unintended Pregnancies (EUP) and contraceptive non-use among women and adolescents in Eswatini.
2. Objective 2 (Health System Capacity) – To evaluate health system readiness, provider knowledge, and attitudes regarding the provision of comprehensive and integrated Post-Abortion Care (PAC) services, including immediate, high-quality Family Planning (FP) counselling and method provision, in both urban (Manzini) and semi-rural (Matsapha) settings.
3. Objective 3 (Barriers and Delays) – To document specific socio-legal and structural barriers, including fear of legal sanction, cost, stigma, and provider refusal, that contribute differentially to delayed access to PAC services across geographic settings (Urban vs. Semi-Rural).

Key Interview with a legal expert from Civic Society

1.6 Methodology

This research employed a sequential exploratory mixed-methods design, integrating both quantitative and qualitative data collection to provide a holistic understanding of the Public Health Triad (GBV, EUP, and PAC). The primary quantitative component consisted of a Comparative Knowledge, Attitude, and Practice (KAP) study for AGYW and another one for community members, while the qualitative component utilized focus group discussions with community members, Key Informant Interviews (KIs) to contextualize the statistical findings through the perspectives of policymakers and civil society leaders

Study Setting

The research was conducted across three strategic sites in Eswatini: Bhunya, Ngonini, and Matsapha. These sites were selected to provide a comparative framework between peri-urban industrial hubs (where young women often migrate for work) and rural settings, allowing the study to capture geographic disparities in service access and social surveillance.

Target Population and Sampling Strategy

The study targeted Adolescent Girls and Young Women (AGYW) aged 15–35, as this demographic is at the highest risk for unintended pregnancy and gender-based violence.

- **Quantitative Sampling** – A non-probability, purposive sampling technique was used to recruit 260 participants from a target of 300. This ensured representation across critical age cohorts, specifically targeting minors (15–17) and young adults (18–26). Also 160 community members filled in a KAP quantitative tool from a target of 200.
- **Qualitative Sampling** – Purposive sampling was used to identify key informants from the SADC Parliamentary Forum, the Ministry of Health, and specialized CSOs (e.g., Women Unlimited,) to provide expert insight into the legal and structural barriers identified in the field.

Data Collection Tools

Data was collected using structured and semi-structured instruments such as KAP Questionnaires which were administered via mobile data collection tools to assess levels of contraceptive knowledge, attitudes toward healthcare providers, and self-reported pregnancy history. A sensitive, validated GBV screening tool was integrated to identify the prevalence of Intimate Partner Violence (IPV) and sexual coercion without re-traumatizing participants. Semi-structured interview guides were used for policy-level stakeholders to explore the Confidentiality Trap and the fiscal implications of unintegrated SRHR services.



Data Analysis Plan

Data was cleaned and analyzed using statistical software for quantitative data using SPSS. Descriptive statistics (percentages, means) were used to establish the prevalence of EUP and GBV. Comparative analysis was conducted to identify significant differences between urban and rural cohorts. Interview and Focus group discussion transcripts were subjected to Thematic Analysis. Codes were developed to identify recurring barriers, such as Professional Hostility and Findings from the KAP surveys were triangulated with policy maker interviews to establish the proxy incidence of unsafe abortion and the Loophole of Inadvertent Aid.

1.7 Study Demographics

Demographics for Quantitative Survey for AGYW

Location	Actual Participants
Total for Urban	196
Total for Rural	64
Total	260

Demographics for Community Members

Location	Actual Participants
Total for Urban	116
Total for Rural	29
Total	145

Focus Group Discussion

District	Number of FDGs Conducted	Number of FDG Respondents
Bhunya	1	21
Ngonini	3* (11+18+19)	48
Matsapha	1	26
Total	5	95

Stakeholders List



Department/Ministry	Total	National	Region and Community level	Village
Department of Social Welfare	1			
Ministry of Health	1			
Ministry of Education	1			
Police	1			
SADC PF	1			
Days For Girls	1			
Women Unlimited Eswatini	1			
Queer Women's Network	1			
SAFAIDS	1			
Traditional Leaders	15			
Health Service Providers- Manzini	16			
Total	40			

1.8 Ethical Considerations and Safety Management

The sensitive nature of research concerning Gender-Based Violence (GBV) and the legal complexities of Post-Abortion Care (PAC) in Eswatini necessitated a high-risk ethical protocol. This study was conducted in strict adherence to the core principles of human rights and participant safety as mandated by national and international research standards.

The study protocol received full ethical clearance from the Eswatini Health and Human Research Review Board (EHRRB). The research was guided by the four pillars of bioethics: respect for autonomy, non-maleficence, beneficence, and justice. Autonomy was prioritized through a comprehensive multi-tiered consent process conducted in the participant's preferred language (Siswati or English). In accordance with the Children's Protection and Welfare Act (2012), a two-tiered consent process was utilized for adolescent participants aged between 15-18 years. This involved obtaining formal parental or guardian consent alongside the minor's personal assent. However, recognizing the high prevalence of intra-familial violence and the risk of reproductive coercion identified in the study rationale, the principle of non-maleficence was given precedence. In specific instances where seeking parental consent posed a documented risk of harm or neglect to the minor, a waiver of parental consent was applied for and granted by the EHRRB, allowing the minor to provide independent informed consent to ensure their safety and participation. Lastly, given the criminalization of certain reproductive health outcomes in Eswatini, strict anonymity was maintained. No personally identifiable information was linked to the Knowledge, Attitude, and Practice (KAP) data. Data management protocols ensured that all sensitive disclosures remained confidential, protecting participants from both social stigma and legal repercussions.

1.9 Study Limitations

The findings of this report must be considered within the context of several structural and data-related limitations that reflect the complex legal and administrative environment of Eswatini.

Paucity of Official Unsafe Abortion Data- A primary limitation of this study is the significant lack of official, state-verified data regarding unsafe abortion. Due to the restrictive legal framework and the criminalization of the procedure, incidents of unsafe abortion are frequently excluded from formal health statistics, leading to a profound data void. To address this, the study utilizes proxy estimates through triangulating high rates of early unintended pregnancy (EUP), documented fear of legal repercussions, and pregnancies resulting from gender-based violence (GBV) to establish the likely prevalence of clandestine procedures.

Reliance on Field-Collected Data and Regional Comparisons – Due to centralized national data being incomplete or unavailable for sensitive SRHR indicators, this study relies heavily on primary data collected directly from the field. This includes Knowledge, Attitude, and Practice (KAP) surveys from 260 participants across urban and rural settings. Where national data was non-existent particularly regarding maternal mortality drivers and the impact of restrictive law regional comparisons from Southern Africa were employed to provide the necessary context and validate the local findings.

Fiscal Cost Challenges and Budgetary Invisibility- The study was unable to establish a precise domestic fiscal cost for the treatment of abortion-related complications because the Eswatini health system does not currently possess a dedicated Budget Code for Reproductive Health or Post-Abortion Care (PAC). This budgetary invisibility makes it difficult to track the exact drain on national health resources. Consequently, the report utilizes regional economic models to establish a fiscal cost framework, highlighting how the state effectively subsidizes the backyard industry by paying for the medical consequences of unsafe procedures that could be mitigated through rights-based policy regulation.

Geographic and Subjective Biases- Finally, the study recognizes a geographic bias toward urban perspectives, as 196 of the 260 participants were based in town-centered environments for the survey for Adolescent girls and young women. However, a significant number of AGYW from rural communities was engaged in other forms of data collection such as FDGs. (See 1.6 for Demographics) Furthermore, the sensitive nature of GBV and abortion means that findings may be affected by social desirability bias, where participants under-report experiences due to high levels of communal stigma and fear of being reported to the police

2. SECTION B: Findings

2.1 Introduction

The findings in this section show the harsh realities facing young women and adolescent girls within a complex context of how violence, unintended pregnancy, and barriers to post abortion care are deeply connected. The data shows that reproductive health is not just about healthcare access; it is tied to widespread gender-based violence and strict laws that punish the most vulnerable thereby hindering available access to SRHR services. This study combines survey data, estimates on unsafe abortion, and expert insights, to highlight how location, legal barriers, and traditional rules deny young women control over their own bodies. These results reveal a public health emergency presented by the absence of standard regulations and policies to provide safe, integrated services thus forcing young women and adolescent girls to seek dangerous, secret care. This results in high costs to both human lives and the economy, threatening the nation's future development and human rights commitments. The outcome also hinders the young women a chance to contribute to the development outcomes of Eswatini effectively.

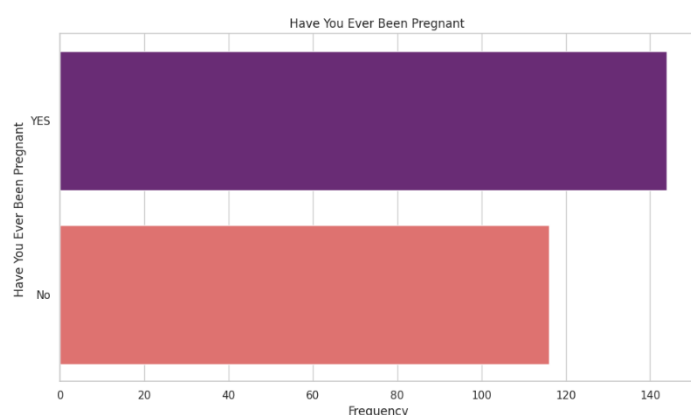
2.2 Objective 1: Situating the Problem

2.2.1 Description of Demographics

The survey data reveals a demographic profile predominantly composed of young women in their late teens and early twenties, with the largest participation coming from the 18 to 22 and 22 to 26 age groups, each representing 80 respondents. A smaller portion of the participants are aged 16 to 18 or 13 to 16, while 56 respondents identified as being in an older category. Regarding geographic distribution, most of the participants reside in urban areas, accounting for 196 individuals, while a minority of 64 respondents are in rural settings. For community members who participated in the survey comprising of men and women, 116 are from rural communities and 29 were from urban communities. For AGYW, this distribution suggests that the findings heavily reflect the experiences and perspectives of young women living in more developed or peri-urban environments. These young women are largely navigating reproductive health within urban settings, which often implies better physical proximity to clinics but, as the data later shows, does not necessarily translate to better SRHR outcomes. The smaller rural cohort provides a necessary contrast, though the overall narrative is heavily influenced by the urban experience.

2.2.2 Pregnancy Prevalence in Group

In terms of reproductive history, a significant portion of the participants have experience with pregnancy, with 144 women reporting they have been pregnant before compared to 116 who have



not. This highlights that maternal reproductive health services are a direct concern for more than half of the group surveyed in this study.

2.2.3 Comparison of Planned vs. Unplanned Pregnancy

The actual figures for planned pregnancies reveal a significant disparity between rural and urban areas.

Among the young women who have ever been pregnant, most of those pregnancies were reported as unplanned.

Location	Total Ever Pregnant	Planned Pregnancy (Yes)	Unplanned Pregnancy (No)
Rural	29	5 (17%)	24 (82.8%)
Urban	115	35 (30.4%)	80 (69.6)
Total	144	40 (27.8)	104 (72.2%)

2.2.4 Operational Definition of EUPs in Study

Age Lens – In this study, Early and Unintended Pregnancy (EUP) is operationally defined through the intersecting lenses of chronological age, legal status, and reproductive autonomy, acknowledging that the implications of pregnancy shift across different stages of development. Primarily, the study identifies early pregnancy as a national emergency occurring among minors under the age of 18, where the term is synonymous with a profound lack of reproductive choice. This is evidenced by the finding that 93.3% of pregnancies in this cohort were unplanned, a reality further complicated by a legal context in which many of these cases constitute survivors of statutory rape. Beyond legal childhood, the definition extends to the 18–24 age cohort; while these individuals can legally consent to sexual activity, pregnancy at this stage is operationally defined as early because it occurs during a critical window for education and personal growth. In this socioeconomic frame, EUPs are viewed as significant barriers to empowerment that hinder young women from completing their schooling or developing the skills necessary to contribute effectively to the broader development outcomes of Eswatini. Ultimately, the study classifies these pregnancies as EUPs whenever the timing of conception undermines a woman's physical, legal, or developmental trajectory.

Unintended Pregnancy (The Autonomy Lens) – Across all age groups, a pregnancy is classified as unintended based on the participant's self-report that the conception was unplanned or undesired at the time it occurred. The study establishes that unintended status is often a symptom of violence

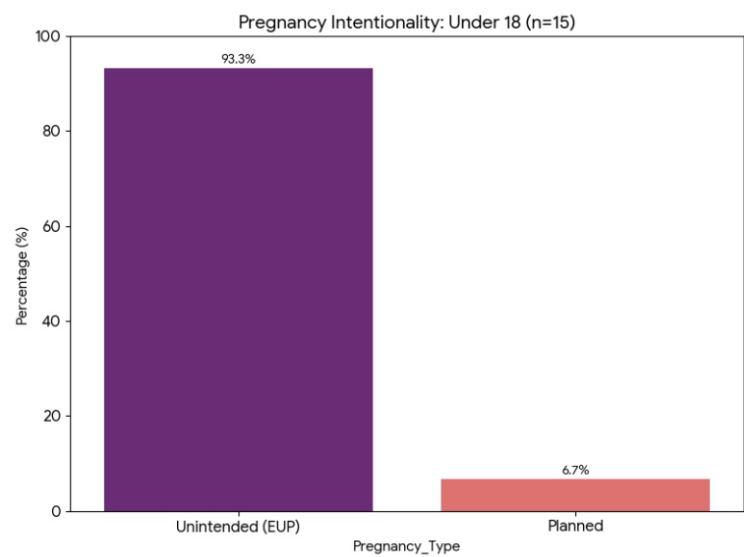
rather than a lack of knowledge. Even in urban centers where contraceptive knowledge reaches 97.3% among adult women, unintended pregnancies remain high, driven by coercive reproductive environments.

The Role of the 24–35 Control Group

To validate these definitions, the study included women aged 24 to 35 as a critical comparison or control group to determine if maturity mitigates these risks. The findings reveal a significant Knowledge–Practice Paradox in which the older cohort exhibits a maturation of knowledge and greater economic stability, nearly half (49%) of their pregnancies remain unintended. The data also shows that maturity does not necessarily grant autonomy; 80% of unplanned pregnancies in this older group occur within the context of Intimate Partner Violence (IPV). In these instances, pregnancies are defined as unintended because reproductive choices such as condom use or contraceptive type are dictated by male partners through physical violence, emotional abuse, or marital rape.

2.2.5 Establishing EUP Prevalence by Age Cohort

Under 18

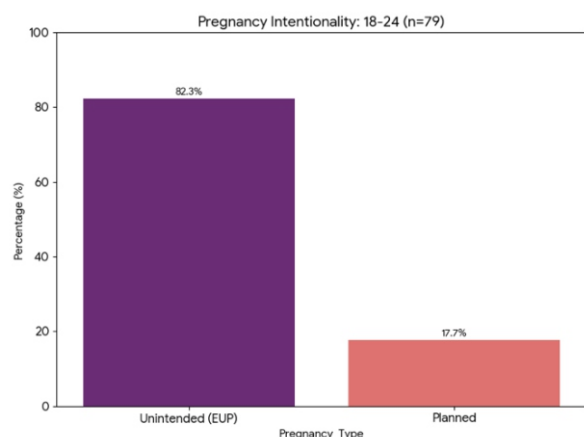


Among the youngest respondents, those under the age of 18, the incidence of unintended pregnancy is exceptionally high³³. For the adolescents in this group who have experienced pregnancy, a staggering 93.3% of those pregnancies were unplanned. This reflects an almost total lack of reproductive planning or choice, highlighting extreme vulnerability and a critical gap in access to effective contraception or reproductive autonomy for girls in their early to midteens. The 6.7%

figure for planned pregnancies is also concerning as it could highlight prevalence of statutory rape, planning for pregnancy in child marriage set up, present of inter generational relationships or planning for pregnancy under duress. An FDG at Bhunya noted that the average age for fall pregnancy debut in their community is usually 15 years.

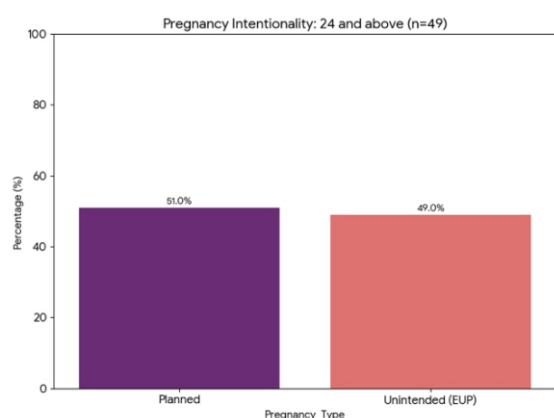
³³ Key Interview with SAFAIDs, Days for Girls,
³⁴ Prevalence of child marriage noted as high in Bhunya FDG

18-24 years of Age



For the transition group, those aged between 18 and 24 years old, the rate of unintended pregnancy remains very high, though slightly lower than among girls under 18. In this age bracket, 82.3% of pregnancies were reported as unintended. In Matsapha, it was observed that this age group largely has 2 to 3 children by the time they get to this age. This suggests that while young adult women are entering a stage of life where they may have more mobility or information on SRHR services, the vast majority are still facing multiple pregnancies that they did not plan or desire to have, pointing to continued systemic barriers to reducing EUPs and consistent contraceptive use.

24 years and above



In contrast, the profile of pregnancy intentionality changes significantly for women aged 24 and above. In this older cohort, the split between planned and unintended pregnancy is nearly even, with 51% of pregnancies being planned and 49% being unintended. This shift indicates that as women mature, they likely gain greater control over their reproductive lives, better economic stability, or more stable relationships that allow for intentional family planning. However, even in this older group, 80% of unplanned pregnancies largely and consistent contraceptive use.

happen within the context of intimate partnership violence. Further probing indicated that men dictate contraceptive choices e.g. refusing to use condoms, telling their partners that if they use pill they won't be "hot" and other contraceptive based myths, physical violence including marital rape, and rape occurring in relationships defined as date rape. In this age range nearly half of all pregnancies are still unintended, showing that reproductive health challenges persist well into adulthood.

³⁵ FDG at Ngonini

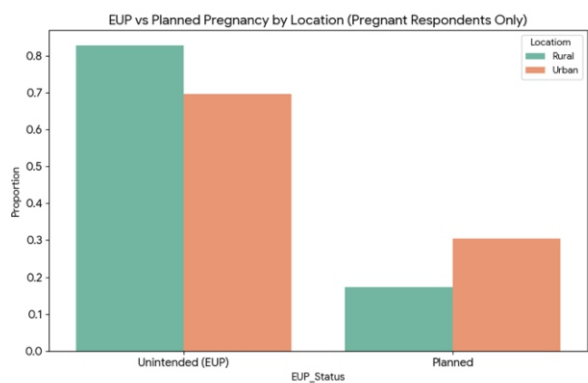
³⁶ FDG with AGYW in Bhunya

³⁷ Ngonini FDG with AGYW

³⁸ FDG at Matsapha

³⁹ Key Informant interview with Women Unlimited Eswatini

2.2.6 EUP vs Planned Pregnancy by location

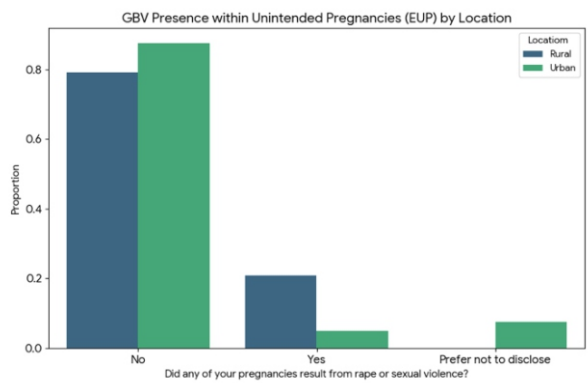


When focusing specifically on those who have experienced pregnancy, the data reveals that most of these pregnancies were unintended, especially in rural settings. Nearly 83% of pregnancies reported by rural respondents were unplanned (EUP), compared to approximately 70% in urban areas. This indicates that unintended pregnancy is a near-universal experience for rural and consistent contraceptive use.

women who become pregnant, while urban women have a slightly higher likelihood of their pregnancies being intentional. For rural women, consultations with traditional leaders and community health workers indicated that contraceptive choices are largely determined by patriarchal influence. Most women seek permission from their male partners to access contraceptives, usage type and largely do not have negotiating power for condom use hence the high prevalence in EUP. ended, showing that reproductive health challenges persist well into adulthood.

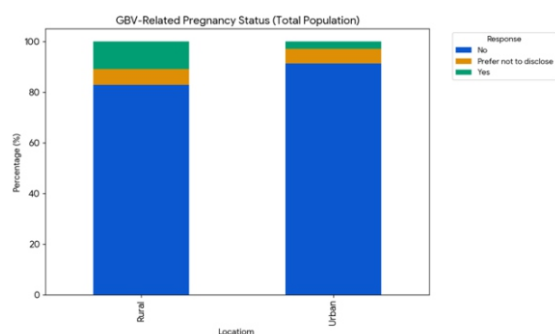
2.2.7 GBV and EUP by Location

Intersection of GBV and Unintended Pregnancy by Location



The intersection of GBV and unintended pregnancy highlights a critical vulnerability for rural cohort. Among rural women who had an unintended pregnancy, more than 20% or 1 in 5 stated that the pregnancy was a result of rape or sexual violence. This is a significantly higher proportion than in urban areas, where only 5% of unintended pregnancies were linked to GBV, with another 7.5% of urban women preferring not to disclose the circumstances.

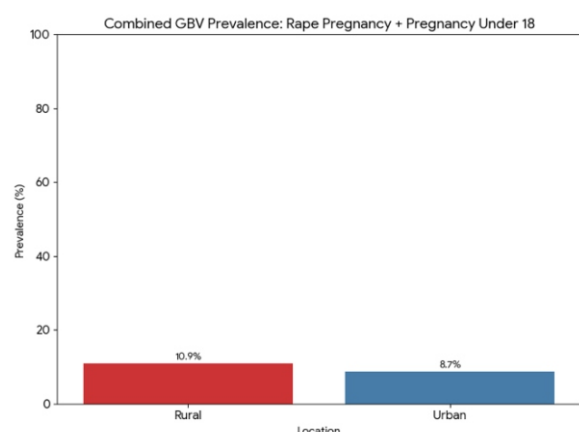
⁴⁰ Consultations with traditional Leaders from Ngonini and Bhunya
⁴¹ Health worker perceptions for those working in rural regions
⁴² FDG Bhunya
⁴³ Interview with Women Unlimited, Key Interview with Department of Social Welfare



In the total population surveyed, in rural Areas 10.9% of all respondents (including those who have never been pregnant) reported having had a pregnancy that resulted from rape or sexual violence. This indicates that at least 1 in 10 young women in rural communities has experienced GBV of such severity that it led to an unintended pregnancy. For urban Areas, the reported prevalence is significantly lower but still present, with 3.1% of all respondents

reporting a rape-related pregnancy. This more than threefold difference between rural and urban settings suggests that rural adolescent girls and young women face a drastically higher risk of reproductive coercion and sexual violence. When looking at the total sample of 260 participants, 13 specific cases of rape-related pregnancy were documented. While this might seem like a small absolute number, in the context of a public health survey, a 10.9% prevalence for a specific type of violent outcome (rape leading to pregnancy) is alarmingly high and points to a critical need for targeted protection and SRHR support services in both urban and rural regions.

2.2.8 Combined GBV Prevalence



To establish a more comprehensive picture of Gender-Based Violence (GBV) prevalence, the study analyzed the intersection of direct rape-related pregnancies and statutory rape proxies (defined as any pregnancy occurring while the respondent is under the age of 18). This combined indicator provides a more realistic estimate of the number of young women whose reproductive lives have been impacted by sexual violence and exploitation. The total figure is composed of two critical groups:

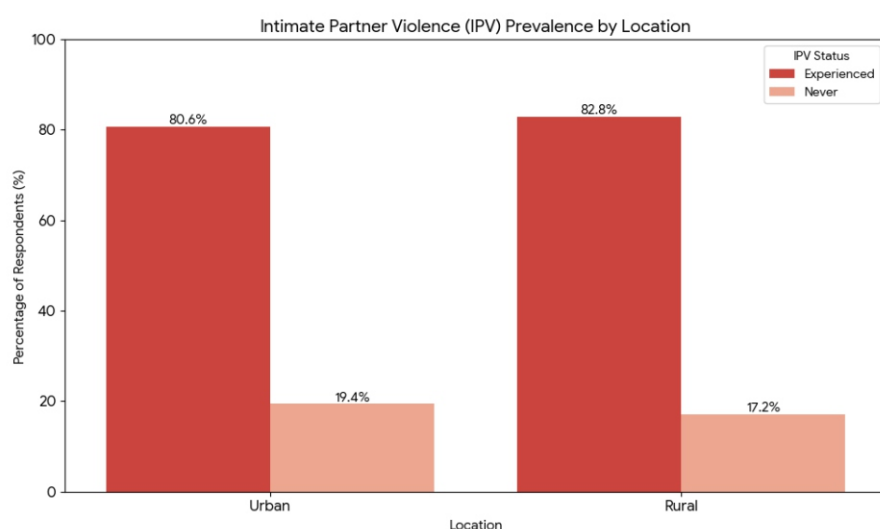
1. **Directly Reported Rape Pregnancies** – 13 individuals who are under 18 explicitly stated their pregnancy resulted from rape or sexual violence.
2. **Statutory Rape Survivors (Pregnancy < 18)** – 15 individuals experienced pregnancy while under the age of 18. While not all of these may have been reported as rape in the survey, they legally constitute statutory rape, representing a failure of protection and a significant SRHR violation.

⁴⁴ Comparison to a total figure of all respondents even those who have never been pregnant to get a general overview of the severity of prevalence of rape and its consequences

3. **Non-overlapping Survivors** –After removing cases where both conditions were met, a total of 24 unique individuals were identified as having their SRHR health severely compromised by sexual violence or underage pregnancy. 21% of all pregnancies were experienced by underage girls.

This data underscores that GBV is not just a secondary factor in unintended pregnancies but a foundational driver of SRHR crises for young women in Eswatini. The higher prevalence in rural areas suggests that rural adolescents are at the greatest risk of sexual exploitation and are the least likely to have the autonomy to prevent its consequences.

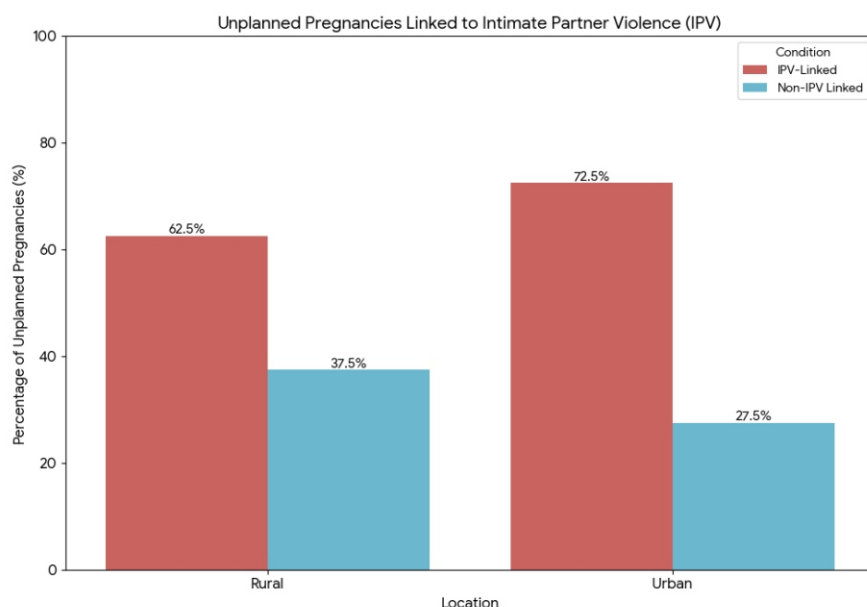
2.2.9 EUP and Intimate Partner Violence



The analysis of Intimate Partner Violence (IPV) reveals an alarming and widespread crisis among Adolescent Girls and Young Women (AGYW) in both urban and rural settings. In combining physical violence, emotional abuse, financial control, and coercive rape, the prevalence of IPV is consistently high across the board. There is high prevalence across both settings, and the data clearly shows that IPV is not a localized issue but a

systemic one. In rural areas, the prevalence is slightly higher, with 82.8% of respondents (53 out of 64) reporting that they have experienced at least one form of IPV. In Urban areas, the rate is nearly identical at 80.6% (158 out of 196). Only a small minority of the 260 respondents reported never experiencing such incidents just 19.4% in urban centers and 17.2% in rural communities.

2.2.10 Intimate Partner Violence and Unplanned Pregnancy Prevalence



The intersection of Intimate Partner Violence (IPV) and unintended pregnancy reveals a profound crisis of reproductive coercion prompting a need to address this challenge through male engagement. When unplanned pregnancies are isolated in analysis for both settings, the majority are found to occur within abusive or coercive relationships. In urban settings, a staggering 72.5% of all unplanned pregnancies (58 out of 80) occurred within a

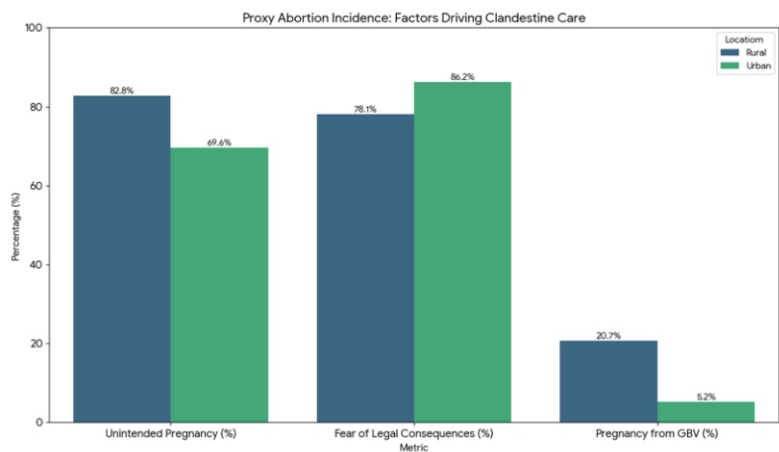
relationship characterized by one form of IPV or another. This high concentration suggests that in urban environments, unplanned pregnancy is frequently not a failure of contraceptive knowledge, but a direct consequence of a lack of bodily autonomy and the presence of partner control. In rural settings, 62.5% of unplanned pregnancies (15 out of 24) were linked to IPV. While the percentage is lower than in urban areas, it still represents most unplanned cases. This indicates that for 3 out of every 5 rural women facing an unintended pregnancy, the situation is complicated by the presence of physical, emotional, or sexual abuse.

The data demonstrates that unintended pregnancy is often a symptom of violence. In relationships where coercion, physical violence, or financial control are present, women are often unable to negotiate the use of condoms or regular contraception. This creates a trap where the violence leads to pregnancy, and the pregnancy in turn increases the woman's vulnerability and dependence on the abusive partner. To this end, reproductive health interventions cannot succeed in a vacuum; they must be integrated with GBV protection services. For a young woman in an IPV relationship, knowing about contraception is insufficient if she does not have the safety to use it. These figures provide a clear explanation for the high proxy incidence of clandestine abortion when a pregnancy is forced through violence or coercion, the drive to terminate that pregnancy often in secret is significantly higher.

Interview with Ministry of Health Official, SADC PF, Ministry of Education
Key Informant Interview with an Official from Department of Social Welfare

2.2.11 EUP and Proxy Abortion Prevalence

Due to the legal restrictions and high levels of stigma surrounding the procedure in Eswatini, the study cannot establish an actual abortion incidence. However, this study presents proxy incidence for abortion by examining the high rates of unintended pregnancies (EUP) and the pervasive fear of seeking post-abortion care (PAC), which together create the conditions where clandestine or unsafe abortions are likely to occur can establish a proxy. This can assist policy makers to design policies with an estimate figure and actual extent to which unsafe abortions are causing SRHR challenges in Eswatini.

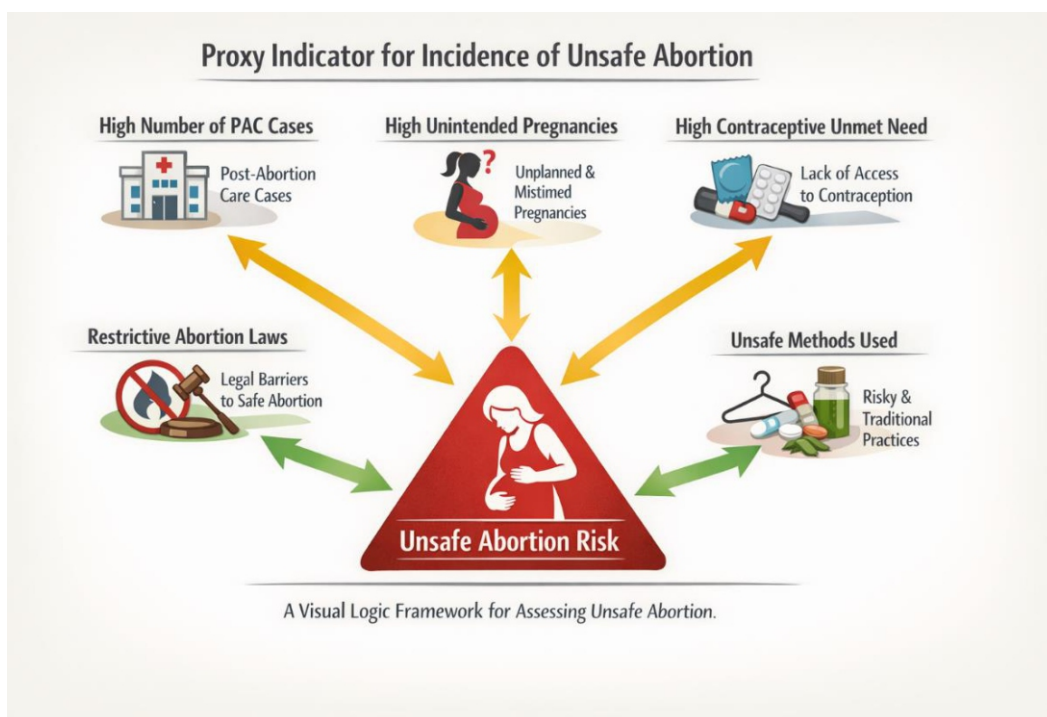


The proxy incidence of abortion is established by looking at three intersecting factors that collectively signal a high likelihood of clandestine or unsafe abortion: high rates of unintended pregnancy, a pervasive fear of legal repercussions, and pregnancies resulting from violence. In rural areas, the Unintended Pregnancy rate among those who have been pregnant is a striking 82.8%, compared

to 69.6% in urban areas. This extremely high prevalence of unplanned pregnancies creates the initial demand for termination services in a legal environment that is largely restrictive. The Fear of Legal Consequences serves as the second major driver of the proxy incidence, acting as a barrier that pushes these unintended pregnancies away from safe, formal healthcare. Surprisingly, this fear is even more acute in urban areas, where 86.2% of respondents believe a woman would avoid a clinic for fear of being reported to the police, compared to 78.1% in rural settings. This widespread confidentiality trap indicates that most women who might seek an

Proxy Abortion prevalence is a methodology used to estimate the frequency of abortions in contexts where legal restrictions or stigma prevent direct reporting. Instead of relying on self-disclosures, it triangulates secondary indicators such as high rates of early unintended pregnancy (EUP), high prevalence of sexual violence, and hospital admissions for incomplete miscarriages to establish a likely trend of unsafe procedures. It does not always require a specific numerical value. While researchers often aim for a proxy number, the methodology is equally effective for mapping severity and risk.

abortion do not view the formal healthcare system as a safe space, suggesting that any terminations occurring are likely hidden.



Finally, the relationship between Gender-Based Violence (GBV) and pregnancy adds a layer of extreme vulnerability to the proxy incidence. In rural areas, 20.7% of women who had an unintended pregnancy reported that it resulted from rape or sexual violence four times the rate of 5.2% found in urban areas. When these three factors high unintended pregnancy, overwhelming fear of the law, and significant rates of GBV-related pregnancies are viewed together, they establish a high proxy incidence of clandestine abortion, where the lack of reproductive control meets a legal and social environment that penalizes seeking help. This translates to a likely high incidence of unsafe abortion in the locations studied.

2.2.12 Unsafe Abortion and Socio-Economic Consequences

The highly restrictive legal environment in Eswatini does not stop the practice of abortion; rather, it makes it inherently dangerous. The abortion rate is estimated at 18.9 per 1,000 women, with roughly 8,800 abortions occurring annually between 2015 and 2019. Because safe, legal services are inaccessible, most of these procedures are clandestine. Methods used for unsafe termination include the insertion of sharp objects or foreign bodies, the ingestion of toxic domestic substances,

["Eswatini country profile". Guttmacher Institute.](#) 2022. Retrieved 2 August 2024.

PAC ranges from US\$3 to US\$114 when induced abdominal trauma. While the use of medications is increasing in the informal markets, improper guidance on dosage or gestational limits, medicinal use frequently leads to incomplete abortions, hemorrhage, and infection, forcing women to seek PAC in a health system that may then report them to authorities if there is sufficient proof that there was an abortion.

The economic burden of unsafe abortion is significant for both contexts under study. Complications drain national health resources through the need for emergency surgery, blood transfusions, and intensive care. For the individual, the consequences include chronic pelvic pain, infertility, and long recovery times that result in loss of income and productivity.²⁶ In the most tragic cases, the lack of options leads to infanticide or the abandonment of newborns in remote areas, a phenomenon frequently reported in local media and noted by judicial officials.

2.2.13 The Loophole of Unintended Aid

The current legal-clinical status quo has created a perverse incentive structure. As is, criminalizing safe abortion but ethically mandating post-abortion care informally, the state has inadvertently subsidized the backyard abortion industry. Unqualified providers operate with near impunity because they know that any medical disaster they create will be cleaned up by the public health system. This loophole effectively means that the government of Eswatini is paying for the consequences of illegal acts that it could prevent through regulation and safe service provision. This adds to the incidence of proxy abortion rates as well.

2.2.13 The fiscal and social cost of unsafe abortion

The economic burden of unsafe abortion on Eswatini's national budget is significant but often remains unquantified in formal health planning. The Kingdom allocates approximately 10% of its budget to health, but reproductive health accounts for only 13% of health expenditures, leaving a heavy reliance on donor funding.

Direct medical and health system costs

The fiscal cost of treating complications from unsafe abortions is exponentially higher than the cost of providing modern contraceptives or safe termination services.

Per-Patient Expenditure – Regional studies indicate that the average per-patient cost of including overhead and capital costs. Incomplete

Unsafe abortion and associated factors among reproductive aged women in Sub-Saharan Africa: a protocol for a systematic review and meta-analysis – PMC, accessed on February 6, 2026,

Ibid, 20

Interview with Health Service Providers

Executive Board of the United Nations Development Programme, the United Nations Population Fund and the United Nations Office for, accessed on February 6, 2026, https://www.unfpa.org/sites/default/files/board-documents/main-document/DP.FPA_CPD_SWZ_8%20-%20Eswatini%20CPD%20-%20Final%20-%202027Jun25.pdf

Direct and opportunity costs of comprehensive abortion care for women living in humanitarian settings in the Amhara Region, Ethiopia – PubMed Central, accessed on February 6, 2026,

The cost of post-abortion care in developing countries: a comparative analysis of four studies – PMC, accessed on



- abortions, which often require surgical intervention and prolonged hospitalization, are significantly more expensive than uncomplicated induced abortions.
- Resource Diversion – Abortions are illegal in Eswatini and have resulted in criminal convictions, but in October 2012 more than 1,000 women were treated for abortion-related complications at a single clinic in the country's second city Manzini, says the Family Life Association of Swaziland (FLAS), a family planning organization. Applying the regional average cost, this single facility's monthly PAC expenditure represents US\$86,000 a substantial unbudgeted drain on public resources.
- Efficiency Gap – One year of modern contraceptive services and supplies costs only 3% to 12% of the average cost of treating a single PAC patient, suggesting that full investment in family planning could yield massive fiscal savings.

Social and private economic costs

Unintended pregnancy and unsafe abortion also impose private costs on individuals and society through lost productivity and the price of inaction.

- Opportunity Costs – For women in humanitarian or high-poverty settings characteristic in rural areas, the opportunity cost of seeking PAC including transportation, lost household income, and caregiver expenses can total nearly US\$200 per case.
- Educational Loss – Girls who discontinue schooling due to EUP face a lifetime of reduced earning potential. An adult mother is three times more likely to complete university than an early mother, and each year of secondary schooling is estimated to significantly reduce the risk of early pregnancy.

Tax Revenue Loss – The State bears a long-term fiscal cost in lost tax revenue due to the lower lifetime income of mothers who began childbearing in adolescence. In regions like

February 6, 2026,

Ibid ,23

Interview with Police, Legal expert

The New Humanitarian, <https://www.thenewhumanitarian.org/news/2012/11/14/illegal-abortion-endangering-lives#:~:text=Abortions%20are%20illegal%20in%20Swaziland,matter%20of%20a%20few%20years>

Estimates of Health Care System Costs of Unsafe Abortion In Africa and Latin America, accessed on February 6, 2026, <https://www.guttmacher.org/journals/ipsrh/2009/estimates-health-care-system-costs-unsafe-abortion-africa-and-latin>

The cost of post-abortion care in developing countries: a comparative analysis of four studies – PMC, accessed on February 6, 2026, <https://pmc.ncbi.nlm.nih.gov/articles/PMC5013781/>

The price of inaction: the global private, fiscal and social costs of children and youth not learning, accessed on February 6, 2026, <https://unesdoc.unesco.org/ark:/48223/pf0000389852.locale=en>

Direct and opportunity costs of comprehensive abortion care for women living in humanitarian settings in the Amhara Region, Ethiopia – PubMed Central, accessed on February 6, 2026, <https://pmc.ncbi.nlm.nih.gov/articles/PMC12590586/>

Interview with official from Ministry of Education

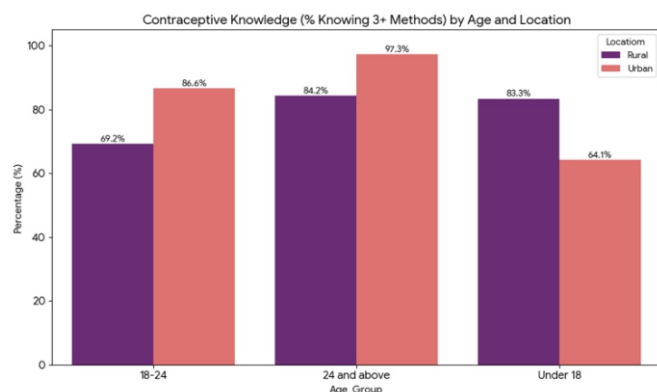
The price of inaction: the global private, fiscal and social costs of children and youth not learning, accessed on February 6, 2026, <https://unesdoc.unesco.org/ark:/48223/pf0000389852.locale=en>

Adolescent pregnancy costs Latin America and the Caribbean USD 15.3 billion a year and deepens inequality, according to new UNFPA report, accessed on February 6, 2026, <https://lac.unfpa.org/es/news/adolescent-pregnancy-cost-latin-america-caribbean>

Latin America, the cost of adolescent pregnancy is estimated at 1% of GDP; a similar projection for Eswatini would suggest a massive drag on national economic growth⁶⁵

2.3. Objective 2: Attitudes and Knowledge

2.3.1 Contraceptive knowledge by age and location



The analysis of contraceptive knowledge defined as knowing at least three reliable ways to prevent pregnancy reveals distinct patterns across age groups and geographic locations.

The Adolescent Knowledge Flip (Under 18)–In a surprising reversal of the adult trend, rural adolescents (83.3%) report higher contraceptive knowledge than urban adolescents (64.1%). This suggests that rural outreach programs or school-based health education in rural areas may be successfully reaching younger girls, or that the higher prevalence of pregnancy in these areas (as seen in previous analyses) has led to an earlier acquisition of SRHR information in local clinics.

The Urban Knowledge Advantage in Adulthood – As women transition into adulthood, urban residents gain a clear advantage. In the 18–24 age group, knowledge in urban areas jumps to 86.6%, compared to 69.2% in rural areas. This may be attributed to rural to urban migration in search for better economic opportunities by the age group. Adding to this, the knowledge which Adolescent girls are getting in high school may not be easy to practice in a patriarchal society where they lose autonomy of their bodies due to the complex nature of relationships, and GBV in the form of intimate partnership violence in which coercive control affects their SRHR aligned health seeking behaviors. Among those 24 and above, knowledge is nearly universal in urban centers at 97.3%, while rural women in the same age bracket lag at 84.2%.

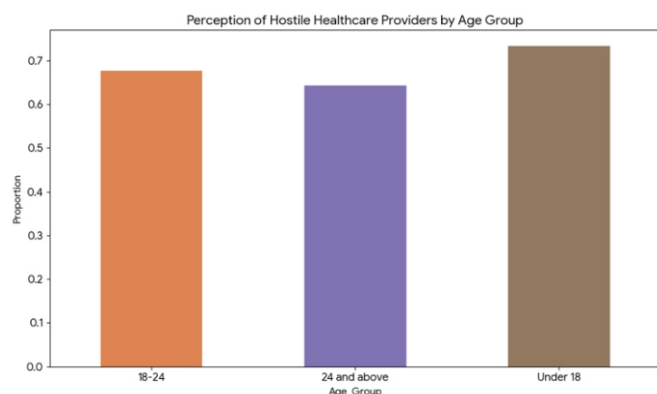
Knowledge Maturity – There is a clear maturation of knowledge over time. In urban settings, knowledge levels climb steadily from 64% (teens) to 97% (adults). In rural settings, while the starting point for teens is higher, the level remains relatively stagnant or fluctuates, peaking at only 84% for older women.

Ibid,32

Key Informant Interview with a Ministry of Education Official

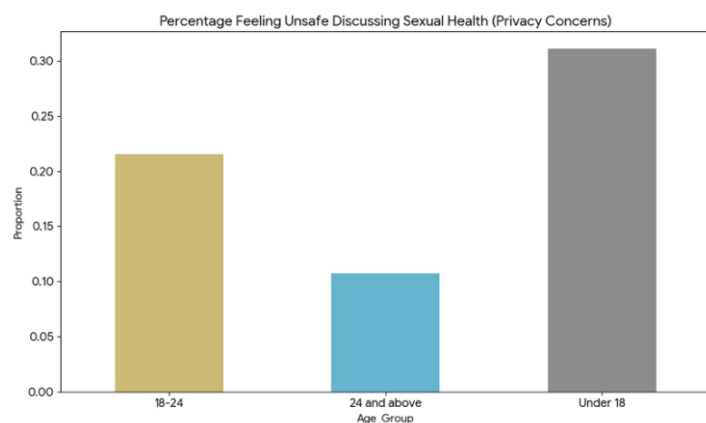
However, these exceptional knowledge rates are not commensurate with the high EUP prevalence and Unsafe abortion rates by either location. This can be attributed to the high rates of coercive reproductive environments as established in the analysis above.

2.3.2 Attitude towards health care providers



Attitudes toward healthcare providers further complicate health-seeking behaviors for the youngest cohort. Over 73% of girls under 18 believe that health workers are judgmental or hostile toward young women seeking reproductive services. This perception of professional hostility is higher among adolescents than in any other age group, creating a powerful psychological barrier that likely prevents them from initiating care even when they know it is necessary.

2.3.3 Privacy Concerns and the Confidentiality Trap

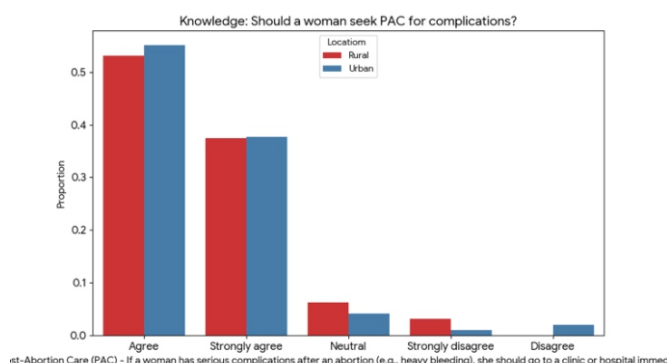


Privacy and confidentiality are also major concerns affecting SRHR health seeking patterns that peak during adolescence. Approximately 31% of girls under 18 either disagree or strongly disagree that it is safe to discuss sexual health issues with healthcare workers without their privacy being violated. This lack of trust in the clinical environment is nearly triple the rate seen in the 24-and-above group (by an additional 11%), indicating that for younger girls, the fear of their sensitive information being shared with

parents or the community is a dominant deterrent to seeking SRHR services. This reveals a profound and dangerous gap between medical knowledge and health-seeking behavior regarding SRHR services or Post-Abortion Care (PAC). While young women understand the life-saving necessity of hospital care for complications, social and legal barriers create a stalling effect that could lead to preventable maternal fatalities. A more profound legal barrier is the Confidentiality Trap, where the legal duty of medical professionals to report suspected injury or illegal acts involving minors under the Children's Protection and Welfare Act (2012) overrides patient confidentiality. Approximately 82% to 87.5% of young women across all

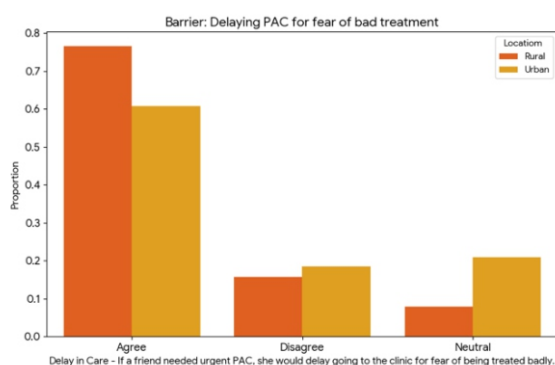
cohorts agree that fear of being reported to the police would prevent them or a friend from seeking care for abortion complications

2.3.4 The Paradox of Knowledge vs. Fear



There is an incredibly high level of health literacy across both rural and urban areas regarding the danger of abortion complications. There is high awareness with 90% of respondents combining Agree and Strongly Agree acknowledge that a woman suffering from heavy bleeding after an abortion should seek immediate hospital care. Despite this knowledge, the data shows that 84% of urban and 78% of rural young women believe a woman would avoid the clinic specifically because of the fear that medical staff will report her to the police.

2.3.5 Fear of Treatment as a Primary Deterrent



The fear of how they will be treated at a health facility is a significant barrier that leads to delays in post abortion-care. Approximately 77% of rural and 61% of urban respondents agree that a friend in need of urgent PAC would delay going to the clinic because she expects to be treated poorly or judged by health workers. Rural young women are significantly more likely to expect delays (77% vs 61% urban), suggesting that the combination of social distance and smaller community dynamics makes the fear of judgmental treatment even more paralyzing in rural settings.

2.3.6 Stigma

The social landscape for adolescents is also defined by a heightened perception of stigma. Nearly 87% of girls under 18 agree that women who have abortions are treated badly by their communities, a level of perceived stigma that is higher than that felt by the 18-24 age group. This atmospheric pressure translates directly into behavioral intentions, as 69% of adolescents state that a friend in need of urgent post-abortion care would likely delay going to the clinic for fear of bad treatment the highest such expectation across all age groups.

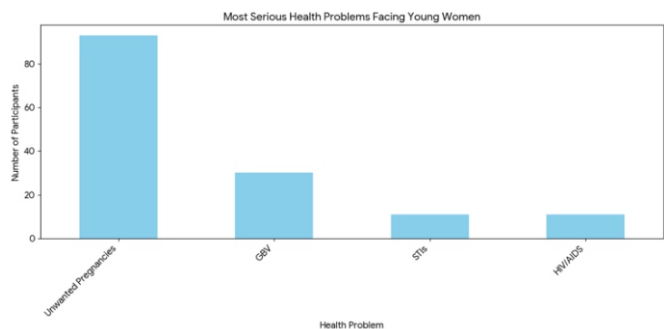
Interestingly, despite these barriers, adolescents acknowledge some strengths in integrated service delivery. Over 73% of girls under 18 agree that those seeking HIV care are reliably offered counseling and contraception, a rate higher than that reported by the 18–24 cohort. This suggests that while adolescents are fearful of initiating care due to stigma and privacy concerns, the healthcare system is relatively effective at providing a package of services once they are already within the facility for other primary needs.

2.3.7 The Triple Stigma of the Marginalized

For women within the LGBTQI community, the barriers are compounded into a Triple Stigma. A representative from the Queer Women's Network points out, a lesbian rape survivor facing an unintended pregnancy is discriminated against for her sexual orientation, her status as a victim of corrective rape, and her desire to abort. This intersectional invisibility means that most queer women avoid formal systems entirely, increasing their risk of death from self-managed, unsafe procedures.

2.4 Objective 3- Barriers to Practice

2.4.1 Most Significant Health Challenge in Communities



The most significant health challenge identified by community members is Unwanted Pregnancies, which was cited by 93 out of the 145 participants. This is followed by GBV (Gender-Based Violence), STIs, and HIV/AIDS.

Based on the survey data, male partners and elders exert significant influence over women's reproductive choices, deeply rooted in cultural and traditional norms. The data reflects a community structure where individual autonomy in sexual health is often secondary to patriarchal or elder approval. A clear majority of the community believes that reproductive health decisions are not solely the woman's choice. Approximately 70% of participants Agree or Strongly Agree that in their culture, a woman should always get permission from her husband or elder before using contraception. The same expectations likely govern relationships outside marriage as

Interview with Queer Women's Network

well in this context. Only about 14% of respondents disagreed with this requirement, indicating that for the vast majority, male partner or elder gatekeeping is a standard cultural expectation.

In terms of influence of traditional and religious leadership, the influence of male figures extends beyond the household to community leaders (who are traditionally male in many contexts within the study area). A massive 93.8% (135 out of 144) of respondents stated that the attitudes of traditional and religious leaders significantly influence what women and girls feel they can or cannot do regarding their sexual health. This suggests that reproductive choices are governed by a hierarchy of influence, first by the husband or elder at home, and secondly by the broader traditional male leadership in the community.

2.4.3 Community Perspectives on abortion

Reproductive choices in Eswatini are heavily mediated by community norms and the attitudes of Traditional and Religious Leaders, who act as powerful gatekeepers. Surveying the broader community reveals a society that remains largely moralistic regarding abortion.

Approximately 63.6% to 75% of community members and elders believe that a woman who seeks an abortion brings shame upon her family. This communal shame creates an environment of profound isolation for women facing unintended pregnancy. Furthermore, 80% to 84% of the community believe that abortion is always illegal in Eswatini, regardless of the reason. This suggests that the conditional legality provided by the 2005 Constitution has not permeated public consciousness. Despite these rigid views, there is a point of latent consensus: 72.7% of respondents agree that health professionals should not face punishment if they provide an abortion to save a woman's life. This highlights a potential pathway for advocacy that frames SRHR through the lens of maternal survival rather than solely individual autonomy.

The influence of Traditional and Religious leaders on women's bodily autonomy is nearly absolute. Over 86% of respondents acknowledge the power of traditional leaders and men over sexual health decisions which women make, with this figure reaching 100% in rural settings. In urban areas, while still high, a larger minority (33.3%) feels these leaders do not significantly influence their health choices. This urban-rural divide is also reflected in trust toward health workers; only 10.3% to 15% of rural respondents trust their local clinics to have the right skills and equipment to treat serious complications, compared to 40% to 53% in urban areas.

2.4.4 Compliance with International Standards

Eswatini's SRHR framework is in direct tension with several international treaties it has ratified. As a state party to the Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW) and the Convention on the Rights of the Child (CRC), Eswatini is obligated to

Reflections from Key Informant Interviews with Traditional Leaders, Focus Group Discussions from Ngonini and Bhunya
Key Interviews with Traditional Leaders, FDG Consultations in Bhunya and Matsapha
Survey results with Community members
Ibid, 20

ensure equal access to health services and protect minors from harm. CEDAW's General Recommendation No. 24 explicitly states that creating legal barriers to health services that only women need, such as safe abortion, constitutes sex discrimination. Regarding the WHO 2022 Abortion Care Guideline, Eswatini is largely non-aligned. The WHO recommends the full decriminalization of abortion and the removal of all grounds-based restrictions, advocating instead that abortion be available upon the request of the woman or girl. Furthermore, the WHO recommends against mandatory waiting periods and third-party authorization barriers that are functionally present in Eswatini through the requirement for multiple doctor certifications or parental consent for minors. Regional compliance with the Maputo Protocol is also a subject of international scrutiny. While Eswatini has ratified the protocol, its laws do not yet protect the full spectrum of reproductive rights mandated by Article 14, such as access to abortion to protect mental health in broader contexts.

International Guideline/Treaty	Eswatini Alignment Level	Primary Conflict Area
WHO 2022 Guidelines	Low ⁷⁷	Mandatory grounds vs. on request recommendation. ⁷⁸
Maputo Protocol Art. 14	Partial ⁷⁹	Failure to authorize for full health grounds. ⁸⁰
CEDAW	Low ⁸¹	Criminalization viewed as sex discrimination. ⁸²
CRC	Variable ⁸³	Conflict between protection and

⁷² THE GLOBAL GAG RULE AND ACCESS TO ABORTION: –Southern Africa Litigation Centre, accessed on February 6, 2026, <https://www.southernafricalitigationcentre.org/wp-content/uploads/2021/02/GGR-Report.pdf>

⁷³ CEDAW General Recommendation No. 24: Article 12 of the Convention (Women and Health) Adopted at the Twentieth Session of the Committee on the Elimination of Discrimination against Women, in 1999(Contained in Document A/54/38/Rev.1, chap. I)

⁷⁴ Advocacy lens: 2022 WHO Abortion Care Guideline–2gether 4 SRHR, accessed on February 6, 2026, https://www.2gether4srhr.org/uploads/files/Resource_Advocacy-lens-2022-WHO-Abortion-Care-Guideline-JALWHO_2022.pdf

⁷⁵ Ibid 27

⁷⁶ Maputo Protocol at 20: Progress on Abortion Rights in Africa, accessed on February 6, 2026, https://reproductiverights.org/wp-content/uploads/2023/08/CRR_Maputo-Protocol_4pg_08292023.pdf

⁷⁷ Eswatini –GAPD – The Global Abortion Policies Database, accessed on February 6, 2026, <https://abortionpolicies.srhr.org/country/eswatini/>

⁷⁸ Advocacy lens: 2022 WHO Abortion Care Guideline–2gether 4 SRHR, accessed on February 6, 2026, https://www.2gether4srhr.org/uploads/files/Resource_Advocacy-lens-2022-WHO-Abortion-Care-Guideline-JALWHO_2022.pdf

⁷⁹ Maputo Protocol at 20: Progress on Abortion Rights in Africa, accessed on February 6, 2026, https://reproductiverights.org/wp-content/uploads/2023/08/CRR_Maputo-Protocol_4pg_08292023.pdf

⁸⁰ Ibid, 32

⁸¹ Eswatini • Abortion in ESA, accessed on February 6, 2026, <https://abortion-esa.section27.org.za/eswatini/>

⁸² THE GLOBAL GAG RULE AND ACCESS TO ABORTION: –Southern Africa Litigation Centre, accessed on February 6, 2026,

⁸³ Ending genderbased violence and all harmful practices UNFPA-Eswatini, accessed on February 6, 2026, <https://eswatini.unfpa.org/en/topics/ending-gender-based-violence-and-all-harmful-practices>



		access for minors. ⁸⁴
SDG 3.1	Off-Track ⁸⁵	MMR far above the target to 70/100,000. ⁸⁶

Conclusions on Findings

The findings in this section converge on a single, sobering conclusion which denotes that Eswatini's SRHR crisis is not driven by a lack of SRHR information alone, but by an interconnected system in which violence, unintended pregnancy, and barriers to post-abortion care reinforce one another and narrow the practical SRHR choices available to adolescent girls and young women. Across settings, unintended pregnancy repeatedly appears less as an individual failure and more as the predictable outcome of constrained autonomy within coercive relationships, where negotiating contraception is unsafe, contested, or socially policed. The research also shows that location matters, not simply in terms of distance to facilities, but in how traditional authority, partner control, and community surveillance shape whether services can be sought at all; even where clinics are physically accessible, fear can make them functionally unreachable. A consistent thread is the paradox of knowledge versus action: many participants understand health risks and recognize when urgent care is needed, yet anticipate judgment, exposure, or punishment if they present for services, producing delays that convert treatable complications into avoidable harm. This confidentiality trap is especially consequential for minors, whose heightened vulnerability is compounded by legal and social dynamics that discourage timely disclosure and care-seeking. Stigma operates as a secondary gatekeeper, isolating those who experience unintended pregnancy or abortion-related complications and intensifying the deterrent effect of hostile or unsupportive provider encounters. For marginalized groups, including queer women and persons with disabilities, layered discrimination further reduces safe pathways into care and increases reliance on hidden, higher-risk options. Taken together, the findings depict a cycle with measurable human and economic consequences: restrictive frameworks and social gatekeeping do not eliminate abortion but shift it into clandestine spaces while transferring the medical, fiscal, and social costs back onto families and the public health system. The evidence therefore points toward an integrated response that treats SRHR, GBV prevention, and respectful, confidential care as inseparable pillars of public health and justice.

⁸⁴ Key Informant Interview with Police, Department of Social Welfare and Ministry of Education

⁸⁵ Executive Board of the United Nations Development Programme, the United Nations Population Fund and the United Nations Office for Women, accessed on February 6, 2026, https://www.unfpa.org/sites/default/files/board_documents/main-document/DP.FPA_CPD_SWZ_8%20-%20Eswatini%20CPD%20-%20Final%20-%202027Jun25.pdf

⁸⁶ Ibid, 38

SECTION C: Recommendations

To address these systemic challenges presented in the findings in Section B, Eswatini has a huge responsibility to be intentional about fixing the burden presented by EUPs, GBV and Unsafe abortion from a collective perspective. To this end, the following evidence-based recommendations are proposed:

3.1 Legislative and Regulatory reform

- Operationalize Section 15(5) through Administrative Guidelines – The Ministry of Health and Ministry of Justice must jointly issue clinical protocols that define the specific procedures for legal abortion clearly. These guidelines must protect providers who act in good faith and mandate that facilities with religious conscientious objectors provide immediate referral to a non-objecting provider. The Standard Operating procedures must integrate a referral pathway for ensuring that women legally seeking abortion services where they qualify follow a pathway that ensures ease of use for services timely.
- Amend Mandatory Reporting Clauses for Post-Abortion Care – Legislative action is required to amend Section 33 of the Children's Protection and Welfare Act. Medical officers should be explicitly granted immunity from the reporting requirement in cases where an adolescent presents with abortion related complications, unless there is clear evidence of ongoing sexual abuse or life-threatening coercion.
- Domesticate SADC Model Laws – The Parliament should prioritize the full domestication of the SADC Model Law on Eradicating Child Marriage and the Model Law on Gender-Based Violence to address the root causes of EUP.

3.2 Health system strengthening and integration

Reducing the intersecting burdens of gender-based violence (GBV), early unintended pregnancy (EUP), and unsafe abortion requires a health system that is accessible, rights-based, and capable of delivering integrated sexual and reproductive health services. Findings from this study demonstrate that gaps in contraceptive access, fear of stigma within health facilities, and uncertainty regarding post-abortion care pathways discourage women particularly adolescents and young women from seeking timely care. Strengthening the health system therefore requires both improvements in service availability and transformation of the quality and culture of care within health facilities. The government must prioritize the revision and domestication of clinical guidelines for Safe Abortion Care and Post-Abortion Care (PAC) to align with World Health

Defines the parameters within which abortion can be legalized and considered in Eswatini

Organization (WHO) international recommendations. These guidelines should focus on the following pillars –

Equitable access to modern contraceptive services – A critical component of health system strengthening is ensuring equitable access to modern contraceptive services. Although awareness of contraception exists among many young women in Eswatini, barriers such as partner control, stigma associated with adolescent sexuality, and limited youth-friendly services continue to undermine effective utilization. Health facilities must therefore guarantee the consistent availability of a full range of contraceptive methods, including both short-acting methods and long-acting reversible contraceptives (LARCs), at primary health care level. Expanding access to implants and intrauterine devices is particularly important for adolescents and young women who face barriers to frequent clinic visits and may struggle with adherence to short-acting methods. Integrating family planning services across multiple entry points including HIV services, antenatal care, and post-abortion care can reduce missed opportunities for prevention and strengthen continuity of care.

Quality and availability of Post-Abortion Care (PAC) – In addition to improving contraceptive access, the quality and availability of Post-Abortion Care (PAC) services must be strengthened as a core component of emergency reproductive healthcare. Post-abortion care is widely recognized as a life-saving intervention that must be available regardless of the legal status of abortion. However, evidence from this study suggests that women frequently delay seeking care due to fears of stigma, judgment, or potential legal repercussions. To address these barriers, the Ministry of Health should prioritize the development and dissemination of clear national clinical guidelines for PAC, aligned with international standards. Such guidelines should standardize the management of abortion complications and miscarriage using evidence-based methods such as manual vacuum aspiration (MVA) and medical management with misoprostol, which are safer and more effective than outdated procedures such as dilation and curettage

Attitudes and practices of healthcare providers – Improving clinical protocols alone is insufficient without addressing the attitudes and practices of healthcare providers. Fear of hostile or judgmental treatment remains a significant deterrent to care-seeking, particularly for adolescents and unmarried women. Health system reforms should therefore incorporate values clarification and attitude transformation training for healthcare providers to promote non-judgmental, confidential, and patient-centered care. Professional associations such as the International Federation of Gynecology and Obstetrics (FIGO) emphasize that respectful care and confidentiality are essential components of high-quality PAC services and play a critical role in

WHO, 2022. Abortion Care Guideline. Geneva: World Health Organization.

Ibid. 89

WHO, 2022. Abortion Care Guideline. Geneva: World Health Organization

FIGO, 2021. Post-Abortion Care: Clinical and Policy Guidelines. London: International Federation of Gynecology and Obstetrics.



encouraging women to seek timely treatment for complications. Strengthening provider capacity through continuous professional development can help shift the culture of care toward one that prioritizes dignity, empathy, and patient safety.

Integrated model of sexual and reproductive health service delivery –The health system should also adopt a fully integrated model of sexual and reproductive health service delivery, in which family planning counselling and voluntary contraceptive provision are offered immediately following treatment for abortion complications. The World Health Organization identifies post-abortion family planning as a key intervention for preventing repeat unintended pregnancies and reducing the recurrence of unsafe abortion (WHO, 2022). Integrating these services within existing health platforms such as HIV and antiretroviral therapy programmes can further improve access in Eswatini's high HIV prevalence context while minimizing stigma associated with seeking reproductive health services.

Health system financing and commodity security – Sustainable improvements in reproductive healthcare also depend on health system financing and commodity security. The government should establish a dedicated and transparent sexual and reproductive health budget line to ensure consistent procurement and distribution of contraceptive commodities and essential PAC supplies. Commodity stock-outs (contraceptives and PAC medication) remain a common barrier to effective family planning programmes across many health systems in sub-Saharan Africa, and reliable supply chains are essential to ensuring that women who seek services can access them without interruption.

Legal and regulatory uncertainties – Finally, health system strengthening must address the legal and regulatory uncertainties that often shape provider behavior in settings with restrictive abortion laws. Many healthcare providers operate with limited clarity regarding the legal provisions governing abortion and the management of abortion-related complications. Developing clear operational protocols that define the clinical responsibilities of providers in delivering post-abortion care would help reduce uncertainty, protect providers from legal risk, and ensure that patients receive timely treatment without fear of denial or delay.

3.3 Community and behavioral change interventions

- **Values-Sensitive Community Dialogue** – Advocacy groups should engage traditional and religious leaders through Reality Mapping exercises that connect the death of young

FIGO, 2021. Post-Abortion Care: Clinical and Policy Guidelines. London: International Federation of Gynecology and Obstetrics.

WHO, 2022. Abortion Care Guideline. Geneva: World Health Organization

African Union Commission, 2018. Maputo Plan of Action 2016–2030 for the Operationalisation of the Continental Policy Framework for Sexual and Reproductive Health and Rights. Addis Ababa: African Union.

- women from unsafe abortion to the loss of community heritage, community development, and economic stability.
- Confidential Digital Health Ecosystems – Develop state-supported, mobile-based SRHR platforms that provide end-to-end guidance (Information, Service Locator, Access) through private channels to bypass social surveillance.

3.4 Male Engagement through Economic Pragmatism

- Tailor SRHR messaging for young men that highlights the economic benefits of planned families and the shared responsibility of pregnancy prevention in a within the precepts of placing a responsibility on them to contribute to the development outcomes of Eswatini from a gendered perspective.
- Develop programs on GBV Prevention which lead in having open and honest conversations on the implications of GBV on the SRHR architecture in Eswatini. This will present a learning point for encouraging men to lead in awareness campaigns on ending GBV as leading driver to GBV.

3.5 Pathways for SRHR Reform in Eswatini

The synthesis of demographic data, legal contradictions, and health system gaps identifies several critical areas for intervention. These pathways for reform must be both legislative and programmatic to be effective.

3.5.1 Legislative Decriminalization and Operationalization

The most profound barrier to safe reproductive health is the threat of criminal sanction.

- Broadening the Ground – Advocacy must focus on expanding the grounds for abortion to include socio-economic factors, Statutory rape and full protection of mental health, aligning with the Maputo Protocol and WHO standards.
- Decriminalization – The goal for maternal safety should be the full decriminalization of abortion, removing it from the penal code and regulating it as a healthcare matter from a rights-based perspective.

3.5.2 Resolving the Confidentiality Trap for Minors

The conflict between the duty of care and the mandate to report must be resolved to protect adolescents.

Clarifying Reporting Mandates for Minors – Clear regulatory guidance is needed to specify that medical officers treating a minor for abortion complications are not required to report



the incident to the police unless there is evidence of active, ongoing abuse. This would separate the medical emergency from the criminal justice system.

- **Autonomous Access** – Policies should be revised to allow adolescents to access SRH services and contraception autonomously, based on their evolving capacities, without the requirement for third-party consent.
- **Provider Sensitization** – National training programs are required to address hostile provider attitudes and ensure that AGYW are treated with dignity and non-judgmental care.¹⁴

3.6 Community Engagement

Given the 89% influence of traditional leadership, socio-cultural change is a prerequisite for improved health outcomes.

- **Dialogue with Traditional Leaders** – Structured engagement with Traditional and Religious Leaders is essential to dismantle stigma. Framing SRHR as a matter of protecting maternal life and ending the burden of GBV can help secure community-level support for reform.

3.7 Action Plan on addressing Challenges presented by EUP,

The recommendations integrated in this action plan were drawn from the recommendations made through key informant interviews by policy makers, traditional leaders, AGYW in FDGs and findings of this study. The result is a comprehensive Action plan relevant to the context of Eswatini.



Annexure: Proposed Integrated SRHR Action Plan for Eswatini

Draft Integrated SRHR Action Plan for Eswatini ⁹⁵

Priority / Timeline	Strategic pathway	Actionable recommendations (what to do)	Key activities / steps (how)	Lead	Co-leads / stakeholders	Key outputs (deliverables)	Suggested indicators (examples)
Immediate (0–12 months)	Governance & coordination of all CSO actors working on ending (EUP/GBV/PAC)	Establish a national multisectoral task force and shared action plan + M&E framework for EUP prevention and PAC/FP integration	Set TORs; harmonize indicators across MoH/MoE/Police/Deputy Prime Minister's office /CSOs; quarterly review meetings; joint annual workplan and scorecard for tracking indicators of progress of the task force	DPMO / Cabinet social cluster	MoH, MoE, MoJ/AG, Police, MoTinkhundla, MoFinance, CANGO, CSO SRHR coalition, youth groups	Task force constituted; national M&E framework; annual costed workplan	Task force active; % actions delivered on time; annual public scorecard published
Immediate	Regulatory	Operationalize	Draft/validate SOPs;	MoH +	Nursing	Signed	# facilities

⁹⁵ The recommendations integrated in this action plan were drawn from the recommendations made by policy makers, traditional leaders, AGYW in FDGs and findings of this study. The result is a comprehensive Action plan relevant to the context of Eswatini.

ate (0–12 months)	reform / operationalization of existing law ⁹⁶	Constitution s15(5) via the development of coordinated MoH–MoJ clinical protocols/SOPs for lawful termination and referral (incl. conscientious objection referral obligations)	provider protection clauses facility referral pathways; SRHR Information dissemination + on site coaching for health service providers on the SOPs	MoJ/AG	Council, EHRRB, hospital management, CSOs (legal/SRHR)	national SOPs & job aids; development of referral directory	with SOPs operating to give timely service for those covered by s15(5); # providers oriented; referral completion rate
Immediate (0–12 months)	Addressing Confidentiality & trust (confidentiality trap) ⁹⁷	Issue clear interim guidance that PAC is life-saving care and clarify reporting thresholds pending legal reform (separate medical emergency from criminal process)	Circulars to facilities; standard PAC intake forms minimizing unnecessary disclosure; staff briefings with Police/Prosecution liaison	MoH	Police, MoJ/AG, hospital leadership, professional councils	National circular/guidance note; facility protocols	% facilities implementing guidance; reduced self-reported fear of reporting (KAP)
Immediate (0–12 months)	Quality of care & respectful services ⁹⁸	Scale up provider sensitization/values clarification to reduce hostile attitudes toward AGYW and key populations when providing services	National training package; integrate into continuous professional development; on-site training for all stakeholders	MoH	Nursing Council, CSOs (Women Unlimited, queer orgs), training institutions	Training curriculum; trained cohorts; client feedback tools	% facilities with trained staff; client satisfaction; reduced reports of stigma

⁹⁶ Recommendation from Policy Makers – Ministry of Health, Department of Social Welfare

⁹⁷ Recommendation from Policy makers – Ministry of Health

⁹⁸ Recommendations from CSOs

			involved (incl. gatekeepers/ support staff/)				
Immediate (0-12 months)	Health system integration ⁹⁹	Establish /FP/PAC “one-stopshop” integrations nobody leaves delivery/PAC/ without FP counselling + method offer	Update service standards; integrate FP commodities into ART sites; discharge checklist; supportive supervision	MoH (SRH/HIV units)	Implementing partners, facility managers	Updated standards; integrated service flow	% PAC clients offered FP; % clients counselled on FP; method uptake post PAC
Immediate (0-12 months)	SRHR commodities security ¹⁰⁰	Create/activate a dedicated SRHR budget code to address SRHR challenges	Budget tagging; monitoring; transparent reporting reporting on uses of funds	MoFinance + MoH	Central Medical Stores stocked with SRHR requirements e.g. contraceptives, Parliament committees, donors	SRHR budget line/code; dashboards	# months of stockouts by method; budget execution rate;
Immediate (0-12 months)	Triple stigma & intersectional inclusion ¹⁰¹	Adopt a Triple Stigma Protocol: specific service standards for LGBTQI persons and persons	Inclusive SOP addenda; disability accessible facilities/info; confidentiality	MoH (incl. Rehab unit)	FODSWA, LGBTQI CSOs, facility management	Inclusive service standards; accessible IEC materials	% facilities with accessible SRHR services; key

⁹⁹ Informed by data collected from AGYW

¹⁰⁰ Recommendation from CSOs Women Unlimited Eswatini, SAFAIDS, and SADC PF, Ministry of Health

¹⁰¹ Recommendation from Queer Women's Network

		with disabilities (PWDs) to end “intersectional invisibility” in addressing SRHR challenges	safeguards; targeted training				population satisfaction; disaggregated reporting introduced
Immediate (0-12 months)	Community engagement & stigma reduction ¹⁰²	Launch values-sensitive community dialogue with traditional/religious leaders (Reality Mapping) linking unsafe abortion deaths to community loss in development.	Chiefdom dialogues; faith leader roundtables; community action pledges; local radio talk shows	MoTinkhundla / Traditional Authorities	MoH health promotion, CANGO, faith councils, women/youth orgs	Dialogue toolkit; community commitment	# dialogues held; attitude shift measures; referrals from community leaders
Immediate (0-12 months)	Male engagement ¹⁰³	Implement male-focused SRHR messaging framed by economic pragmatism and shared responsibility	Youth forums; workplaces; sports clubs; couple counselling prompts	MoH + MoYouth	CSOs, traditional leaders, schools	Male engagement toolkit	Condom/FP supportive attitudes; reported shared decision making
Immediate (0-12 months)	Evidence on informal/unsafe abortion market	Conduct a rapid assessment of informal abortion pathways (pills/providers/referral patterns) to inform risk mitigation and	Mixed-method mapping; facility PAC pattern review; anonymized community informant interviews	MoH (research) / EHHRRB	CSOs, academics, Police liaison	Assessment report; risk mitigation recommendations	Completed study; policy brief produced; response plan adopted

¹⁰² Recommendations from Consultations with Traditional Leaders

¹⁰³ Recommendations from CSOs + Data from Survey findings from both rural and urban communities

		law/policy responses					
Medium (1 –2 years)	Legislative reform mandatory reporting ¹⁰⁴	Amend Child Protection and Welfare Act s33 ¹⁰⁵ to provide immunity/exe mption for clinicians providing PAC to adolescents unless clea r evidence of ongoing abuse/coercion	Draft amendment; consultations; parliamentary process; implementation guidance	Parliament + MoJ/AG	MoH, DPMO, Police, CSOs	Amended statute, implementing regulations	Amendment passed; reduced fear of reporting; increased PAC uptake timeliness
Medium (1 –2 years)	Law reform architecture ¹⁰⁶	Establish/activate a Law Reform Commission/committee to align outdated statutes (e.g., CP&E Act 1938) with current SRHR realities	Formal mandate; technical drafting support; schedule of priority reviews	MoJ/AG + Parliament	Legal practitioners, Police, MoH, CSOs	Law reform roadmap; draft bills	Commission formed; # laws reviewed; # reforms tabled
Medium (1 –2 years)	School-clinic linkages ¹⁰⁷	Fully disseminate and implement SRH referral guidelines for schools; strengthen counsellor/nurse referral pathways	Train headteachers/career guidance; referral forms + feedback loops; safe transport pathways	MoE + MoH	Regional education offices, school health nurses, CSOs	Operational referral mechanism nationwide	# schools using referral tool; referral completion; time-to- service
Medium (1 –2 years)	Comprehensive Sexuality Education	Strengthen age- appropriate comprehensive	Curriculum refresh; teacher training; safeguarding	MoE	MoH, DPMO, CSOs, disability orgs	Updated curriculum content;	CSE coverage; knowledge

¹⁰⁴ Recommendations from Policy makers

¹⁰⁵ Children's Protection and Welfare Act

¹⁰⁶ Recommendation from Sipho Gumede, Legal Expert, Representative from the Police

¹⁰⁷ Recommendation from the Ministry of Education

	safeguarding ¹⁰⁸	sexuality education and GBV prevention (incl. consent) in curriculum; ensure inclusion of boy child and PWD	protocols; parent engagement			trained teachers	gains; reported GBV incidents addressed
Medium (1–2 years)	GBV justice response ¹⁰⁹	Improve case processing speed and survivor-centered justice (delays, retaliation fears), especially for minors and PWD survivors	Specialized court processes; victim support services; police/prosecution training; safe reporting channels	MoJ + Police	DPMO, MoH (clinical forensic), CSOs	Strengthened GBV case management	Case turnaround time; conviction rates; survivor satisfaction
Medium (1–2 years)	Fiscal sustainability & joint financing ¹¹⁰	Promote joint funding proposals and pooled programming to reduce fragmentation and competition	Common concept notes; shared budgets; consortium agreements	MoFinance / Donor coordination + CSO coalition	Donors, UN agencies, NGOs	Joint proposals; pooled initiatives	# joint proposals funded; % funding aligned to national plan
Medium (1–2 years)	Community-led and youth-led solutions ¹¹¹	Expand flexible funding + capacity building for youth/community-led SRHR initiatives, including queer and PWD voices	Small grants; mentorship; governance training; M&E coaching	CSO coalition / CANGO	Donors, MoYouth, traditional leaders for support	Youth-led projects scaled	# youth initiatives funded; reach; outcome indicators
Long-	Policy alignment	Domesticate SADC	Draft domestication	Parliament	SADC PF,	Domesticated	Laws

¹⁰⁸ Recommendation from SADC PF Representative, Ministry of health

¹⁰⁹ Recommendation from CSOs, Police, Sipho Gumedze Legal Expert

¹¹⁰ Recommendation from SADC PF Representative, Women Unlimited Representative

¹¹¹ Recommendations from CSOs – Days for Girls, Department of Social Welfare

term (3+ years)	with regional/international standards ¹¹²	Model Laws on GBV and Child Marriage; strengthen enforcement to address drivers of EUP upstream	bills; implementation plans; resourcing		MoJ, MoH, MoE, DPMO	laws + implementation frameworks	enacted; budgeted implementation; prevalence trends
Long-term (3+ years)	Abortion law reform pathway ¹¹³	Pursue staged reform: broaden lawful grounds (incl. mental health/socio-economic/rights based) and move toward decriminalization as a health matter	Multi-year advocacy; evidence briefs; national dialogue; legislative drafting	Parliament + MoJ/AG	MoH, CSOs, traditional/religious leaders, SADC PF	Reform bills; public consensus pathway	Reforms tabled/passed; reduced unsafe abortion complications
Long-term (3+ years)	Digital SRHR ecosystems ¹¹⁴	Launch state-supported confidential mobile SRHR platform (information + service locator + access pathways)	Design with privacy; link to clinics; multilingual content; hotline integration	MoH + Ministry of ICT	Youth orgs, CSOs, telecoms	Live platform + referral network	Users reached; referrals completed; privacy/safety audits
Long-term (3+ years)	Resilience, poverty & climate linkage ¹¹⁵	Integrate SRHR into social protection and climate resilience planning to mitigate poverty-driven vulnerability	Targeted support for vulnerable households; school retention supports; food security linkages	DPMO + MoTinkhundla	MoAgriculture, MoE, MoH, social welfare, donors	Integrated resilience plan elements	School dropout reduction; transactional sex risk reduction proxies

¹¹² Recommendations from SADC PF Representative

¹¹³ Recommendations from Women Unlimited Eswatini

¹¹⁴ Recommendations from CSGsSAFAIDs, Days for Girls

¹¹⁵ Recommendations from SADC PF Representative